

BUDGET EXECUTION IN HEALTH: FROM BOTTLENECKS TO SOLUTIONS



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Budget execution in health: from bottlenecks to solutions

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EXECUTIVE SUMMARY

Budget execution is essential for achieving Universal Health Coverage. Poor budget execution undermines a health system's ability to deliver timely, effective, and equitable care.

Health budgets are designed to finance the procurement of drugs and supplies, pay for health workers' wages and salaries, and cover operational and maintenance expenditures.

Delays and lapses in executing these budgets can therefore leave a health system vulnerable to stock outs, a disengaged workforce and deteriorating infrastructure. It also makes it difficult to advocate for increases in health budget allocations going forward.

From a health service delivery perspective, when service providers cannot fully access and use public funds in a timely manner, their ability to deliver health services according to needs is jeopardized.

Significant efforts have been made to understand and advocate for greater budget allocations in health, the execution of these budgets often remains overlooked.

This joint report by the World Health Organization and the World Bank aims to address this gap by (i) providing conceptual clarity on how budget execution relates to health; (ii) presenting trends and patterns

in budget execution data; (iii) identifying root causes of problems and policy options for reform; and (iv) offering a diagnostic approach that supports operational engagement.

The report situates budget execution within the public finance and health financing literature. It defines budget execution in health as “the degree to which money has been spent in line with agreed priorities and in support of effective health service delivery”, thereby emphasizing the importance that a budget be fully implemented and - equally importantly - that resources are well spent. Health sector financing priorities and generic public finance objectives should align and be mutually supportive. For example, effective budget execution protocols in health should enable a public finance system that supports expenditure control and health financing principles such as output-based payments and provider autonomy.

The report presents budget execution data trends and patterns from 91 countries across various income groups and regions. Analysis of this data identifies the following six challenges:

1. **Underspending: There is significant underspending in low- and middle-income countries (LMICs).** On average, LMICs underspend their health budgets by 13 percent, representing an average loss of about \$4 per capita annually. This is equivalent to what many countries spend on primary care and translates into a missed opportunity to achieve key health outcomes.
2. **Declining execution rates: Budget execution rates have been falling in low-income countries (LICs), especially in Africa.** On average, LIC budget execution rates declined by 1.6 percentage points annually between 2010 and 2020.
3. **Overspending: There is evidence of overspending, primarily in high-income countries (HICs).** This suggests different public financial management (PFM) shortcomings than those found in budget under-execution, often related to the strength of and compliance with controls and oversight mechanisms.
4. **Execution variability: There is considerable variability in budget execution rates over time.** Execution rates in many countries vary significantly, sometimes by as much as 15-20 percentage points year on year. Variation is greater in LICs with poor PFM capacity, undermining their ability to pursue strategic policy priorities.
5. **Poor execution compared to other sectors: Execution rates are worse in health than in the education sector.** This difference is most pronounced in LICs, averaging 7 percentage points.
6. **Under-execution of the goods and services budget: Countries are under-executing the goods and services budget, leaving staff without necessary supplies to deliver services.** Wage and salary payments are typically close to fully executed in all countries. In contrast, in low- and lower-middle-income countries, spending on the goods and services budget that covers medical supplies is about 15 percentage points below that, compromising the delivery of quality services.

What drives poor budget execution in health, and how can these issues be effectively tackled? The report systematically maps out the underlying factors that lead to poor budget execution by budget cycle stage and responsible agency. Reasons for poor budget execution relate to:

- Budget credibility;
- Budget structure and reallocation rules;
- Fund flow regularity and predictability;
- Spending controls;
- Procurement and contract management;
- Budget monitoring system and accountability mechanisms.

Under each of these headline categories the report offers a detailed analysis with concrete examples of how budget execution problems may come to be and how these relate to various agencies in government. Identifying what role ministries of finance, health and local government can play allows the report to offer pragmatic guidance on what action can be taken to address specific problems in budget implementation.

The report also presents a practical and user-friendly diagnostic tool to help countries identify budget execution challenges and engage in a reform agenda. The four-step diagnostic approach guides countries to:

1. Map current funding flows and understand their regulatory PFM landscape;
2. Estimate aggregated and disaggregated budget execution rates to provide a quantitative understanding of current performance;
3. Systematically identify the underlying factors contributing to budget execution challenges; and
4. Develop tailored policy actions to address the identified root causes and sub-causes of inadequate execution.

Finally, this report serves as a call to action, as its findings have far-reaching implications for achieving Universal Health Coverage.

Effective and efficient health budget execution is not merely a technical issue; it is a fundamental pillar for ensuring equitable

access to quality healthcare for all. Simply advocating for good expenditure management is insufficient. Improving health budget execution **requires a holistic approach**, including the strengthening of **budget formulation and oversight functions**, as well as implementing a more agile and responsive **PFM system**.

To enable this, proactive dialogue and collaboration are needed among finance and health authorities, as well as with local governments in decentralized contexts.

Countries that execute their health budgets well may be better positioned to advocate for increased budget allocations going forward. **Countries are encouraged to generate and publicize increasingly granular expenditure data to enhance the visibility of this agenda and enable more systematic analysis.** It is time for LMICs to move beyond the status quo and embrace a more strategic and collaborative approach to health budget execution. By implementing the approach outlined in this report, governments can unlock the full potential of their health budgets, ensuring that every dollar invested translates into tangible improvements in the health and well-being of their populations.

ABBREVIATIONS

AFR African Region

AMR Region of the Americas

EGP Electronic Government
Procurement

EMR Eastern Mediterranean Region

EUR European Region

HCOs Health Care Organizations

IFMIS Integrated Financial Information
System

IMF International Monetary Fund

LG Local Government

LIC Low Income Country

LMIC Lower Middle-Income Country

MDA Ministries, Departments
and Agencies

MHIF Mandatory Health Insurance Fund

MOF Ministry of Finance

MOH Ministry of Health

NHI National Health Insurance

OTIM Online Treasury and Investment
Management System

PBB Program-Based Budgeting

PBS Public Budget System

PEFA Public Expenditure and Financial
Accountability

PERs Public Expenditure Reviews

PFM Public Financial Management

PMG Programme of Medical Guarantees

RMNCAH Reproductive, Maternal, Newborn,
Child and Adolescent Health

SEAR South-East Region

TSA Treasury Single Account

UHC Universal Health Coverage

UMIC Upper Middle-Income Country

WHO World Health Organization

WPR Western Pacific Region

NAVIGATING THE REPORT

This report builds on a joint WHO-World Bank initiative on budget execution challenges (described in more detail in **Appendix 1**).

Following the introduction, **section 2** provides an overview of core concepts from both a public financial management perspective and a health financing perspective and discusses how budget execution challenges can differ in countries with a separate purchasing agency, varying degrees of decentralization and significant donor funding.

The trends and patterns in health budget execution data are analyzed and discussed in **section 3**. The analytical approach is detailed in **Appendix 2**.

Section 4 identifies challenges that hinder good budget execution in health. These are drawn from an extensive literature review and a set of case studies. A summary of the case studies is provided in **Appendix 3**.

Policy responses to these challenges are offered in **section 5**. This section lays out a pragmatic approach that Ministries of Finance, Ministries of Health and local governments can take to address six of the most frequently encountered budget execution challenges and points to examples from cases studies.

Section 6 offers an engagement approach to address budget execution challenges. A four-step diagnostic-to-solution model is proposed that guides an analyst through the process of identifying the right questions, data, and policy options in a structured manner.

Section 7 concludes with a call to action for countries to embrace a more purposeful and collaborative budget implementation approach that involves finance, health and local government stakeholders.

The compiled dataset used for the analysis is being released jointly with this report to support complementary analytical efforts.

FOREWORD

Expediting progress toward universal health coverage requires commensurate resources. Resources need to be raised and budgets allocated to meet the growing challenges in the health sector. However, budgets are only as useful as they also are implemented. This report “***Budget Execution in Health: From Bottlenecks to Solutions***” assesses budget execution performance in low and middle-income countries. It demonstrates that under-execution is widespread and examines where key challenges lie and what drives them. It also proposes practical solutions to strengthen implementation.

The study is timely and relevant given the growing competition on public resources and recognition that progress towards UHC will require more and better utilized public financing. Fully- and appropriately executed health budgets strengthen the case for greater allocations in the future. Conversely, a poorly implemented budget will be the first stumbling block in advocacy for dedicating more resources to health. This report highlights the importance of aligning budget structures with service delivery needs, strengthening financial

management arrangements, and the need for improved communication and coordination among ministries of finance, health and local government agencies.

This report is a valuable resource for policymakers, health finance practitioners, and development partners. It can be used as an evidence base to benchmark performance and help identify challenges. It can also serve as the foundation for a reform program that ensures public finance mechanisms are in place that facilitate access to quality services for all.

We call on governments to prioritize this agenda. Governments can carry out a thorough diagnostic using the approach proposed here to identify actions that health, finance, and local government authorities can take. Timely, granular and high-quality budget execution data are critical to this process; we therefore advocate that data is shared to strengthen transparency and guide action. The problem will be better tackled once it becomes more visible. The WHO and World Bank are fully committed to support countries’ efforts.

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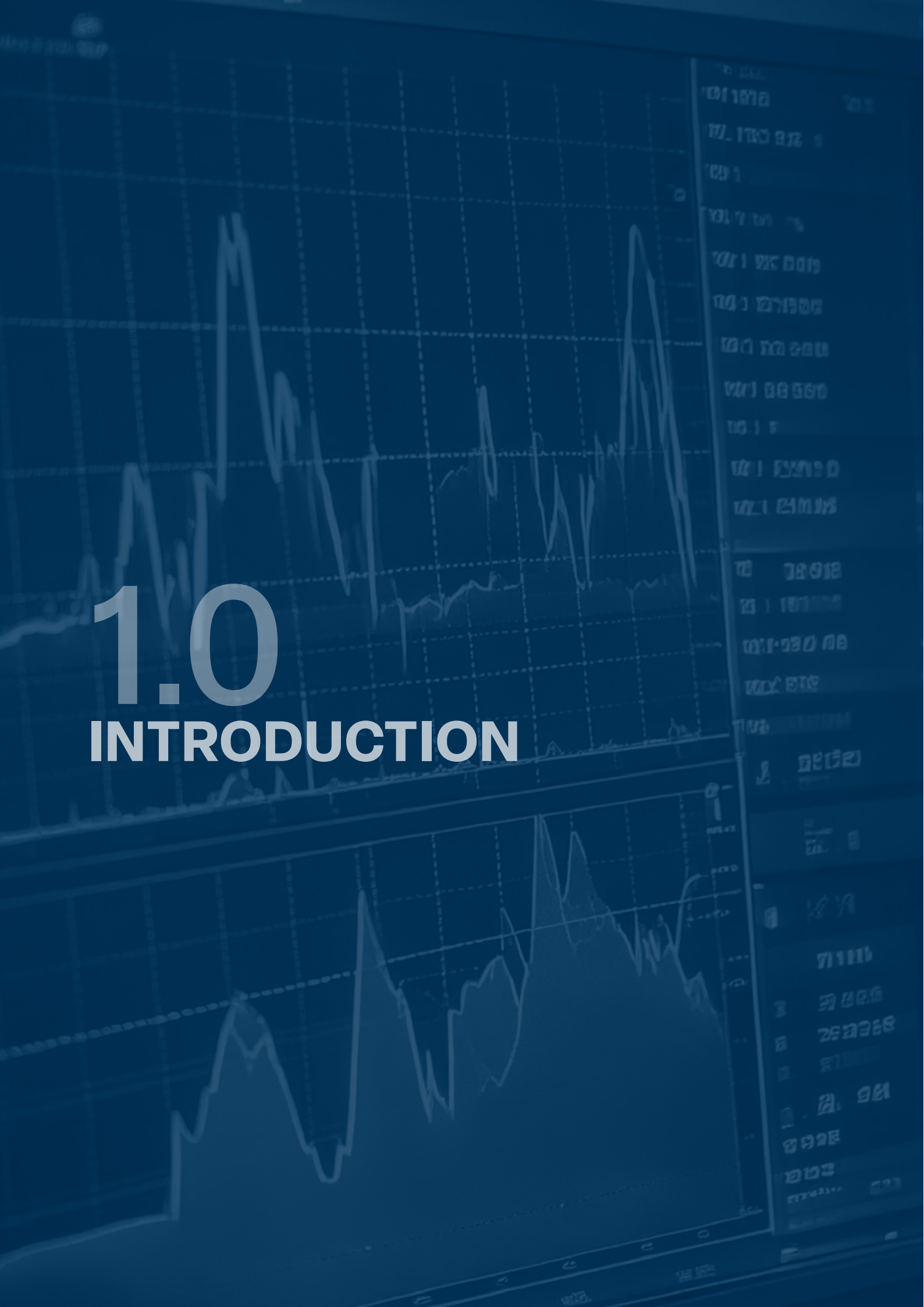
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The background of the slide is a dark blue gradient. It features two overlapping financial line charts. The top chart shows a highly volatile line with several sharp peaks and troughs. The bottom chart shows a line that is generally upward-trending but with more moderate fluctuations. Both charts are plotted on a grid of dashed white lines. On the right side of the image, there is a vertical list of text in Hebrew, which appears to be a stock market ticker or a list of financial data points. The text is white and somewhat faded, blending into the blue background.

1.0

INTRODUCTION

Making progress towards universal health coverage (UHC) requires that budgets be executed effectively and efficiently. Budget execution is the phase of the budget cycle in which the budget is implemented. **This phase consists of authorizing and transmitting funds to budget holders who in turn manage and account for expenditures.** Budget execution is critical to enabling effective purchasing of health services: when service providers are not able to fully access and use public funds in a timely manner, it jeopardizes their ability to deliver health services in line with health needs. Even when a separate purchasing agency is responsible for purchasing health services, budget execution processes remain crucial to ensure timely budget transfers to purchasing agents, to subsidize priority population groups or services, and to provide supply-side support to service providers.

Despite its crucial role in health spending and service delivery, assessing and addressing shortcomings in budget execution have typically not been priorities in low- and middle-income countries

(LMICs). LMIC government officials generally have paid more attention to mobilizing and allocating health-related revenues than to health budget execution. This is partly due to challenges in compiling and publicizing health budget execution data and the health community's limited understanding of budget execution-related processes, which together have resulted in health budget execution's low visibility both within LMICs and globally.

This report aims to deepen understanding of common challenges in health budget execution, identify reform options and facilitate mutual appreciation and collaboration amongst finance, health and local government authorities. The health sector's budget execution data have not yet been widely analyzed in the literature, nor have findings been leveraged to inform policy actions. The report also aims to provide an analytical framework to support future assessments of health budget execution challenges in countries. It was produced through a joint WHO-World Bank initiative that is presented in more detail in Appendix 1.



A person with glasses and a ponytail is sitting at a desk, looking at a computer monitor. They are holding a pen in their right hand. The desk has a mouse, a calculator, and some papers. The background is a blurred office setting.

2.0

**WHAT IS BUDGET
EXECUTION?**

This section aims to clarify the definition of budget execution, as well as how budget execution relates to the health sector. It first discusses the budget execution process from a generic public financial management (PFM) perspective. It then reflects on how that process relates to the health sector, and the additional factors that should be considered to ensure the process supports health financing and health care service delivery effectively. Lastly, the section addresses how to think about the relationship between health budget execution challenges and different purchasing arrangements, degrees of decentralization, and donor financing modalities.

2.1 Definitions and concepts

As a key part of the budget cycle, budget execution is the phase where resources are used to implement the policies that are expressed in the budget (Allen and Tommasi 2001). In its most simplified form, a budget needs to be sequentially (i) formulated and approved; (ii) executed; and (iii) evaluated, to inform the next budget cycle. During the first phase, the line Ministry (e.g., a Ministry of Health) develops a budget proposal, the executive negotiates the budget proposal internally, and the executive submits its agreed budget to the legislature for approval. The government expresses its policy priorities within the budget, which then becomes the executive's legally binding spending plan for the coming year. The next phase, budget execution, is when the budget is implemented and funds are spent, allowing health service providers to deliver services.

In the literature on PFM, budget execution covers a sequence of activities which

operationalize the use of public resources in a way that should be efficient, effective and enable aggregate expenditure control.

The payment cycle is the process that operationalizes budget execution. Budget execution includes several sub-steps, from how funds are authorized and how they flow to budget holders, to how expenditure is managed and accounted for. In most LMICs the budget execution process includes stages for authorization (ministries are authorized to spend money in a manner consistent with the legal appropriations for each line item), commitment (where a future obligation to pay is incurred), verification (verification that goods have been delivered or the service has been rendered), payment authorization (where a payment order is issued), payment (where the payment happens), and accounting (where transactions are recorded, allowing for reconciliation).

If designed and implemented well, these stages collectively facilitate the joint objectives of expenditure control and effective and efficient use of resources (Potter and Diamond 1999).

Budget execution performance is generally assessed through budget implementation rates. The PEFA Framework for Assessing Public Financial Management (PEFA 2019) provides guidance on how to assess the extent to which a budget has been executed (called “expenditure outturn”) relative to the originally approved amount (Table 1.1). Using PEFA’s four-letter scoring scale, a country which deviates from its original budget by a maximum of 5 percent in terms of aggregate expenditure and budget composition outturns, is considered a top performer (“A”). Executing the budget with a deviation beyond 15 percent of the original budget is considered poor performance (“D”). The aggregate budget execution rate can be broken down by economic category,

functional classification, or program classification to offer insight into the extent to which approved budgets have been implemented relative to the intended spending categories.

Spending may be compared against the original budget (as recommended in the PEFA Framework), the revised budget, an adjusted budget, or any subsequent iterations thereof.

Table 2.1
Main PEFA indicators for budget execution

Score	Minimum requirements for scores
Aggregate expenditure outturn	
A	Aggregate expenditure outturn was between 95% and 105% of the approved aggregate budgeted expenditure in at least two of the last three years
B	Aggregate expenditure outturn was between 90% and 110% of the approved aggregate budgeted expenditure in at least two of the last three years
C	Aggregate expenditure outturn was between 85% and 115% of the approved aggregate budgeted expenditure in at least two of the last three years
D	Performance is less than required for a C score
Expenditure composition outturn by function	
A	Variance in expenditure composition by program, administration or functional classification was less than 5% in at least two of the last three years
B	Variance in expenditure composition by program, administration or functional classification was less than 10% in at least two of the last three years
C	Variance in expenditure composition by program, administration or functional classification was less than 15% in at least two of the last three years
D	Performance is less than required for a C score
Expenditure composition outturn by economic type	
A	Variance in expenditure composition by economic classification was less than 5% in at least two of the last three years
B	Variance in expenditure composition by economic classification was less than 10% in at least two of the last three years
C	Variance in expenditure composition by economic classification was less than 15% in at least two of the last three years
D	Performance is less than required for a C score

Source: PEFA 2019.

Budget execution is closely linked to other stages of the budget cycle. Budget execution needs to be treated as a dynamic process that draws from, and feeds into, other stages of the budget cycle and involves all necessary stakeholders including ministries of finance, health and local government (figure 2.1). Effective budget execution requires a well-formulated budget that matches sector needs to

available resources. A budget that does not reflect **sectoral priorities**, is not in line with **strategic plans**, or that aims to achieve **unrealistic goals** will be difficult to execute. Similarly, a budget that is not based on realistic revenue projections is unlikely to be implemented in full. Additionally, failure to adequately monitor a budget can distort the budget execution status, artificially lowering or increasing execution rates.

Figure 2.1

Effective budget execution in health follows good management across the budget cycle by all engaged stakeholders



Source: Authors

Budget execution in health can be defined as the degree to which money has been spent in line with agreed priorities and in support of effective health service delivery.

This definition emphasizes the importance of **executing the budget in full and spending it well**. While budget execution's generic definition and underlying principles apply equally to health as they do to other sectors and to broader government spending, **there remain some specificities in health**.

Compliance with approved budget allocations matters in health as it does in other sectors, with an additional need to be able to match resources with service needs. Given that **health needs commonly change within a fiscal year**, and the **difficulty in anticipating these changes** and estimating budget allocations accordingly, governments should design their budget execution practices **to enable budget holders**, including health service providers, where relevant, to **flexibly respond to evolving**

population needs. Sections of this report unpack the factors that drive health budget execution so as to better inform future assessments and policy responses to be better tailored to the health sector's specificities.

Strategic purchasing and budget execution are interrelated concepts, but they are not the same and should not be conflated when discussing health spending. The purchasing of health services aligns closely with budget execution in situations where a purchasing agency in government makes a payment to a service provider for the delivery of individual health services. In situations where a service provider has the status of a budget entity within the government, the relationship is less direct because the amount that each service provider receives is determined earlier, during the budget formulation phase, and is not linked to outputs. Instead, in this situation, budget execution stage activities consist of service providers using their budget allotment to pay suppliers for inputs. **Budget execution in health commonly goes beyond making payments to service providers**—including spending for other budgetary categories such as **centrally-managed program expenditures** (for example, for prevention), **large procurement contracts for drugs and supplies** managed centrally and executed through large framework contracts, capital expenditures, and even **personnel expenditures**. Therefore, while components of purchasing overlap with budget execution, strategic purchasing cuts across all stages of the budget cycle.

How budget execution relates to health varies according to country context and types of entities involved in health financing. Countries with a separate purchasing agency may face different types of budget execution challenges

than those without. Similarly, the extent to which a country is decentralized and how this shapes health service financing, will affect how budget execution relates to the sector. Lastly, the presence of donor funding and the extent to which donors make (or do not make) use of government PFM systems and institutions may affect budget execution. These considerations are discussed in more detail in the following subsections.

2.2 Health budget execution in countries with different purchasing arrangements

Health budget execution processes are shaped by a country's purchasing configuration, specifically whether there is a dedicated purchasing agency and how that agency engages with public budgets.

The types of budget execution challenges may differ depending on whether (i) the health ministry uses its regular budget to purchase services; (ii) there is a separate purchasing agency, but its budget is part of the regular budget; (iii) there is an extra-budgetary entity tasked to purchase health services, under separate budget rules (Allen and Radev 2010). The differentiation between budget execution and service purchasing processes is outlined in Box 2.1.

When services are purchased through the regular budget, regular budget execution rules apply. Budget execution challenges, such as budget underspending or inefficient use of budget resources by budget holders, may emerge due to **generic weaknesses in a country's PFM's system**, for example affecting

Box 2.1:

Differentiating budget execution and service purchasing processes

Service purchasing, which generally refers to paying service providers based on performance information, and budget execution are interrelated concepts, but they are not the same and should not be conflated when discussing health spending. The purchasing of health services aligns closely with budget execution processes in situations where a government makes a payment (otherwise considered a budget transfer in the PFM taxonomy) to a service provider for the delivery of individual health services. In situations where a service provider has the status of a budget entity within the government, the relationship is less direct because the amount that each service provider receives is determined earlier, during the budget formulation phase. Instead, in this situation, budget execution stage activities consist of service providers using their budget allotment to pay suppliers for inputs, which constitute a payment. Additionally, budget execution in health commonly goes beyond making payments to service providers –including spending for other budgetary categories such as centrally-managed program expenditures (for example, for prevention), large procurement contracts for drugs and supplies managed centrally and executed through large framework contracts, capital expenditures, and even personnel expenditures. Therefore, while components of purchasing overlap with budget execution, strategic purchasing cuts across all stages of the budget cycle.

fund release timing or providing insufficient flexibility in budgetary resource use. If service providers do not have recognized status as spending units this can further complicate the budget execution process and add layers to the process of disbursement, which in turn can exacerbate underspending. Having expenditure reporting as part of the health ministry's general accounting framework helps to ensure relatively comprehensive and public transparency on spending.

When there is a purchasing agency under the administrative structure of the ministry of health, regular budget rules may apply, with some possible adaptations. The purchasing agency may receive budget allocations from the health ministry's regular budget, generally as

a single budget line. Delays in budget transfers to the purchasing agency can hinder effective budget execution, and ultimately service delivery (Schieber et al 2012). The purchasing agency generally can pay providers directly by transferring funds; the latter are budgeted and treated as expenditures when disbursed and recorded accordingly. As the purchasing agency itself provides and consolidates financial reporting, it may not provide disaggregated information on spending by service providers.

A purchasing agency which does not draw on the regular budget generally follows a separate set of processes and rules for budget allocation and execution. For example, the purchaser may be allowed to retain revenue across fiscal years and follow different spending



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arrangements, often with a higher degree of flexibility than under general budget rules. The purchaser may still receive general budget transfers to cover certain population groups/ services, in which case delays in funds transfer will affect the purchaser's ability to spend. Contracts between the purchaser and service providers, which are separate from general budget execution, typically regulate the modality of how the service providers are paid and the level and type of financial autonomy they have. In such cases, the purchasing agency generally must provide separate reports, which are then reviewed and publicized separately from expenditures under the general budget.

2.3 Decentralization and health budget execution

Decentralization influences how budget execution in health operates. In a system which is devolved or deconcentrated, health budget execution is more complex, given the

multiple sources of funding and the often-weak financial management capacities at subnational levels (Ravishankar et al, 2024). Budget execution performance can be affected by multiple layers, authorization requirements, and accounting processes.

In devolved contexts, where administrative, fiscal, and political authority are shifted from the central government to a lower level of government, budget execution processes are more complex than in centralized systems; this complexity can contribute to poorer performance. The lower level of government

– for example, a province, a district, or a municipality – becomes responsible for financing and providing various public services, including healthcare. If the central-level government delivers inter-governmental transfers late or does not execute them fully, it impedes the lower-level government's ability to spend in accordance with plans. The likelihood of such delays or partial transfers is higher when the central level transfers funds originating from multiple grants and funding sources.

Subnational governments' often weaker financial management capacities, and health facilities' financial autonomy limitations, may also affect the ability to fully execute the budget. Unfinished devolution transition may also make spending protocols and processes more complex due to lack of clear responsibilities. For example, subnational governments may be granted formal administrative responsibility for primary care service delivery and considered politically responsible for service outcomes – however they are not granted full fiscal control over the budget for delivering these services, making budget execution fragmented and uncoordinated. Additionally, subnational governments tend not to report their financial transactions as frequently or as comprehensively as central-level governments, and may use different financial management systems, affecting financial accountability, including for health.

Deconcentration, under which local areas are vested with authority and resources while operating as branches of the central ministry, presents specific budget execution challenges. The central-level ministry provides budget allocations which the lower-level entities then execute. The districts' often-limited flexibility over the use of those resources, combined with their limited financial management capacities, can lead to poor budget execution levels. The central government can directly fund lower-level healthcare facilities, either through direct transfers to the facilities' financial management systems or via “pass through” grants to the facilities' parent-districts. When those facilities

are recognized as “spending units” in the government's financial management system, then service provider spending is reported against the regular central budget; when they are not considered “spending units”, it has often been harder to match resources with needs and enable full execution.

2.4 Donor financing and health budget execution

The modality, volume, and predictability of external financing for a country's health sector¹ can impact domestic health budget execution performance. Development partners commonly use a wide range of modalities to transfer aid resources to recipient-country governments. Each modality has implications for the execution of domestic budgets (Piatti-Fünfkirchen et al. 2021a).

External resources in the form of general or sectoral budget support can facilitate good budget execution. With general budget support, a development partner directly transfers external funds to the consolidated account of the recipient country's treasury. These resources are fungible and their receipt is often conditional on the government's policy actions. This modality may improve predictability of the government's revenue and cash-flow position. Additionally, the policy-based conditionalities attached to the development partner's budget support can enable government reforms strengthening domestic budget execution practices. If

1 In LMICs, external sources represented 31 percent of current health expenditure on average in 2021 (WHO, Global Health Expenditure Database, 2023).

expected resources do not materialize in full, however, this can leave part of the budget unfunded and make it harder to completely execute.

Sectoral budget support in health limits the government to using those resources for the health sector, and can enable better practices in the sector. This support may have specific reporting requirements or be linked to certain policy conditions or results, and the greater scrutiny that this entails is likely to have a positive effect on how the government executes its health budget. As these funds are typically channeled through the government treasury and follow standard government PFM practices, generally no additional rules, regulations, or procedures are needed. However, providing the required reports necessitates a greater level of administrative effort. As with general budget support, if anticipated sectoral budget support does not fully materialize, the government may face an unexpected financing gap.

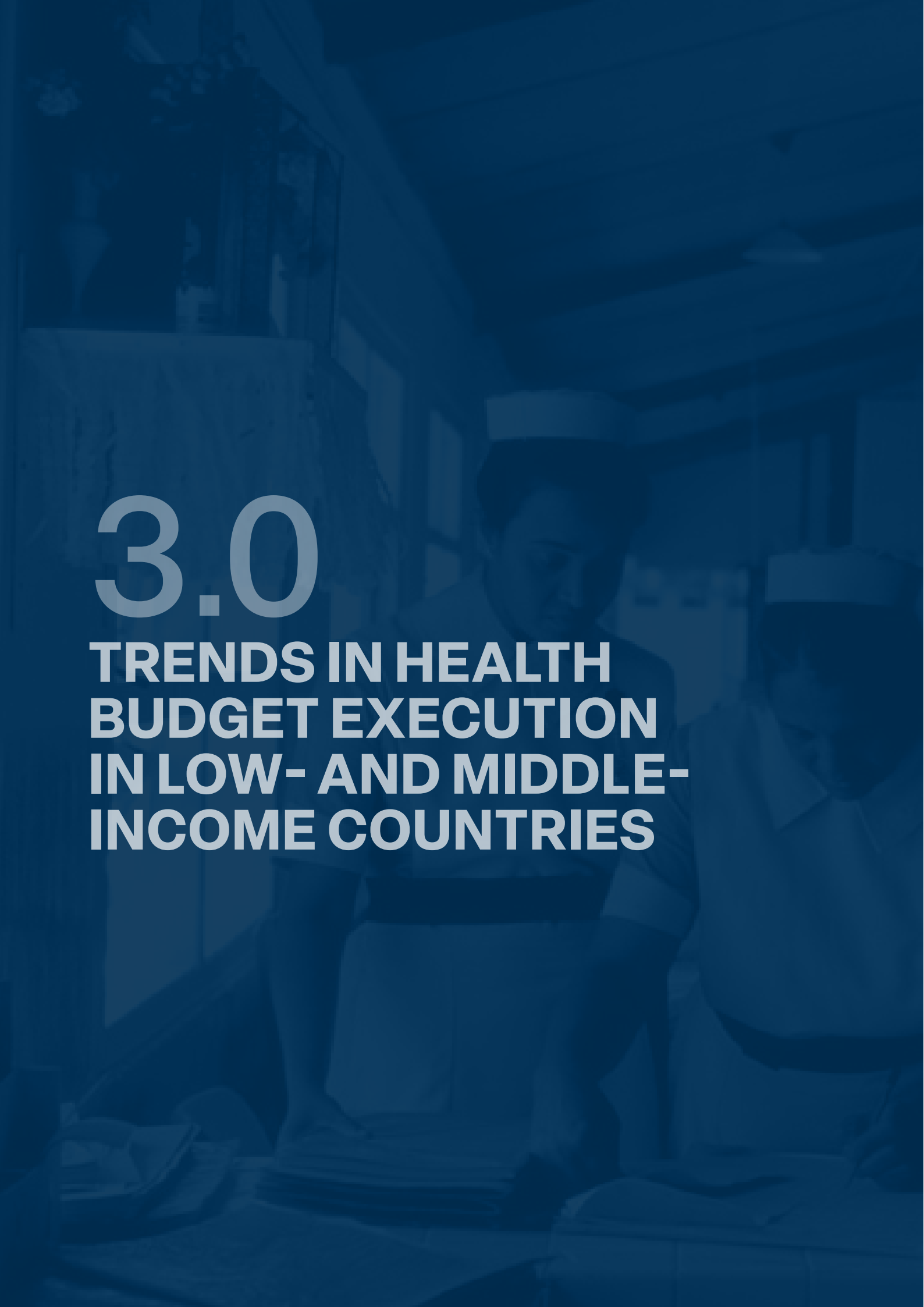
External financing that is earmarked to specific interventions or budget lines can make domestic budget execution more complex. This modality comes with specific financial management rules and conditions and requirements for spending controls, which together can create additional rigidities

on top of the inflexible budget execution protocols often in place in many LMICs.² Even when funds are channeled through the budget, this tends to be through the development budget where it is possible to have separate accounts, rules, and processes in place, which can lead to complications. Donors may insist on dedicated financial reporting on their expenditure items, which may require the recipient government to provide more detailed and frequent reporting, potentially involving the development of customized reports within its financial management information system. Earmarking resources for specific purposes (for example, mosquito net provision, drug or vaccine payments, health worker salary contributions, or capital investments) may help to ensure that these objectives are financed and implemented. In practice, identifying, tracking, and reporting on earmarked spending requires significant effort³ and can draw scarce capacity away from other tasks that are important in regular budget funding management. Nonetheless, while greater budget execution oversight increases the government's immediate administrative burden, it can potentially support higher standards of financial management in the long term.

2 Donors may require that certain conditions be applied to transactions funded by their resources. These can include regulations such as more stringent rules for procurement; a “no objection” approval system for large-value transactions; lower limits on delegated expenditure resulting in more frequent need for senior-level approval of spending; or positive confirmation that each expenditure transaction is being used to finance only eligible expenditures. Donors may also require governments to manage day-to-day expenditure using dedicated accounting systems even when the spending is on budget, an obligation which adds cost and time to budget execution.

3 Donors are concerned that the resources they provide be used prudently and efficiently. Consequently, donors often establish extra regulations for monitoring and auditing the specific spending areas they fund, such as additional financial audits, no-notice audits of specific institutions, forensic audits of high-value transactions, and performance audits for specific areas of the relevant sector (in this case, health).





3.0

**TRENDS IN HEALTH
BUDGET EXECUTION
IN LOW- AND MIDDLE-
INCOME COUNTRIES**

In this section, we report key findings from an analysis of Ministry of Health (hereafter referred to as health) budget execution rates across a sample of 91 low- and middle-income countries over the period 2010-2020.

The analysis shows key trends in health budget execution over time across the three country-income groupings and within individual countries, important distributional patterns by budget component, and compares health budget execution levels with the countries' general government and education budgets. The analytical approach is further detailed in Appendix II. The dataset is made publicly available in conjunction with this report to enable further assessments.

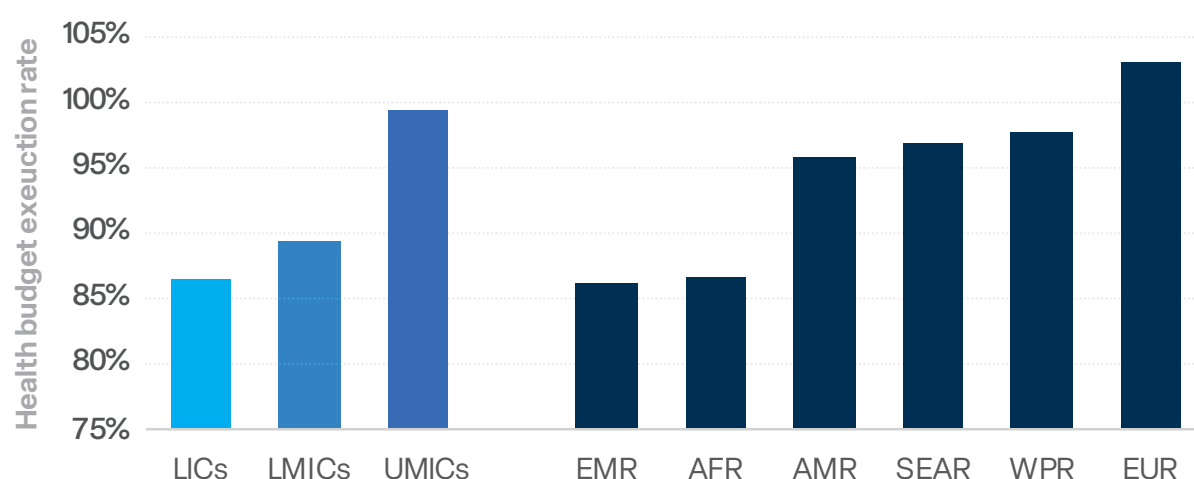
3.1 Health budget under-execution in the 19 low-income countries represented an annual loss of about Intl US\$4 per capita

Health budget execution rates were poor in the sampled low-income countries.

Average (mean) health budget execution rates ranged from 87 percent in the low-income countries to 99 percent in the upper-middle-income countries over the time-period (Figure 3.1). The low-income countries' deviation of 13 percentage points from full budget execution (that is, 100 percent) represented significant resource loss, estimated at Int \$4 per capita per year (constant 2020 international dollars).

Figure 3.1

Low-income countries and those in the WHO African region and the eastern Mediterranean region underspent their health budgets
(mean by group for 2010-2020)



Note: Loss of resources intended to be available for health was calculated based on WHO Global Health Expenditure Database (last accessed August 2023) by taking LICs' average per capita expenditure in constant (2020) PPP over the 2010-2020 time-period and dividing it by the average budget execution rate to estimate full budget execution; the difference between full and actual budget execution represented the loss.

Across regions, health budget execution rates were lowest in the WHO African and Eastern Mediterranean Region countries, with average rates as low as 87 percent and 86 percent, respectively. These regions have the highest number of fragile, conflict-affected, and vulnerable settings where health budget execution volatility is more marked. (Figure 3.1). The region with the highest budget execution rates was the European Region, with an average rate of 103 percent (a slight overspend) for the time-period. There, sustainability of health spending was the greater concern (OECD 2015).

3.2 Health budget execution rates deteriorated over time in low-income countries especially across the Africa Region

Health budget execution rates deteriorated over time, particularly in the low-income countries (Figure 3.2a) and across the WHO African Region (Figure 3.2b). The low-income countries' health budget execution rates fluctuated year-over-year, with a general downward trajectory over time. The average annual change of -1.6 percentage points over the 11-year period indicates that budget execution was a chronic issue. By contrast, rates for the lower-middle-income countries and the

upper-middle-income countries generally hovered around 90 percent and 100 percent (the latter equivalent to full execution) respectively, over that period.

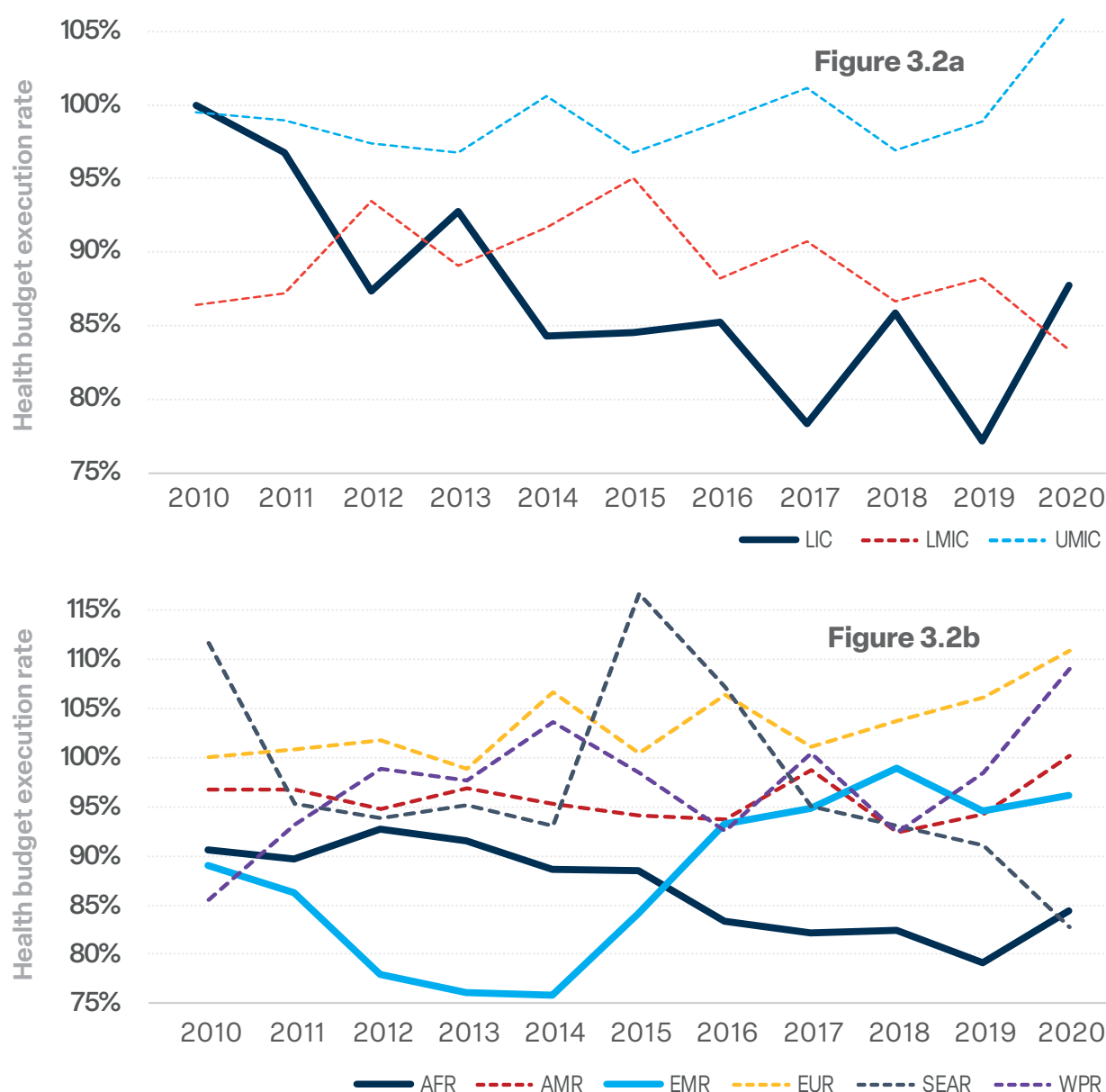
Budget execution rates declined over the course of the time-period for countries in the WHO African Region while generally fluctuating between 95 percent and 105 percent in the other regions (Figure 3.2b). Countries in the WHO Eastern Mediterranean Region (solid green line) were an exception – their rates declined markedly between 2010 and 2012 during the Arab Spring, flatlined the following two years, then began to rise in 2014 coinciding with reforms to strengthen the overall economy and public institutions (Fouad 2013, Charaoui 2023), subsequently converging with countries of the four better-performing regions. All regions except for the WHO South-East Region increased their rates between 2019 and 2020, most likely associated with simplified spending processes during COVID-19 (WHO, 2021).⁴

Low-income countries were, on average, challenged by both considerable under-execution of their health budgets and by a general decline in execution rates over time. Figure 3.3 shows individual low-income countries' execution rates in a quadrant chart. The vertical axis represents the average health budget execution rate for the study period, such that countries shown above (below) the horizontal green line, on average, overspent (underspent) their health budgets.

4 The increase in 2020 was related to spending to address the COVID-19 pandemic (Kurowski et al. 2023) – many countries implemented emergency spending measures, drew on contingency funds, revised public finance laws, exercised flexibility in allocations across the budget, and streamlined their spending modalities. These measures, which contributed to higher budget execution, suggest that countries can overcome bottlenecks in their PFM systems (WHO 2022; Barroy et al. 2020; Hsu et al. 2022).

Figure 3.2a and 3.2b

Health budget execution rates deteriorated in LICs and WHO African region (mean by group from 2010-2020)

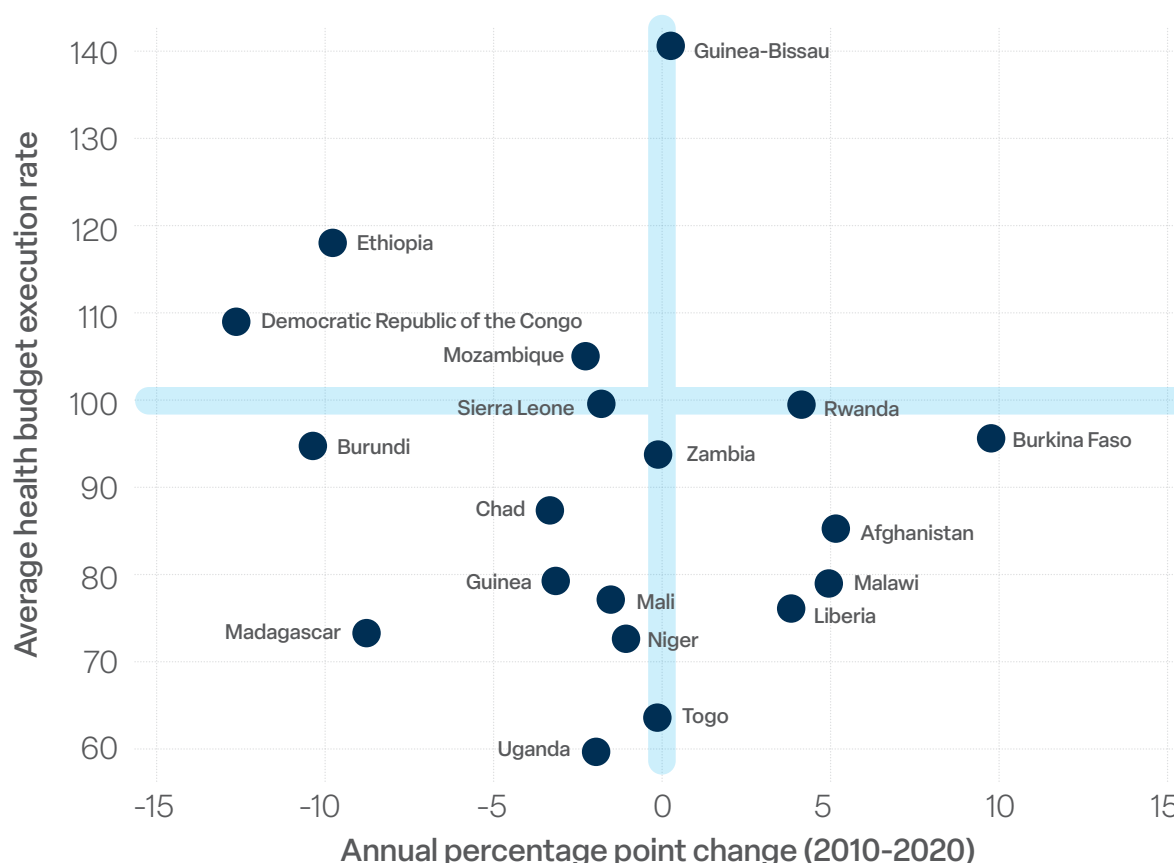


The horizontal axis represents the average annual percentage point change over the time period, such that countries to the right of the vertical line showed positive (negative) changes over the time-period. The countries in the bottom-right quadrant tended to

underspend their health budgets while showing budget execution improvement. Countries in the bottom-left quadrant also tended to underspend their health budgets, but with worsening budget execution; in these countries, more focused attention is needed.

Figure 3.3

In many of the low-income countries budget execution rates were both low and worsening over time



Note: Average annual percentage-point change estimated by regressing health budget execution rates by year of data.

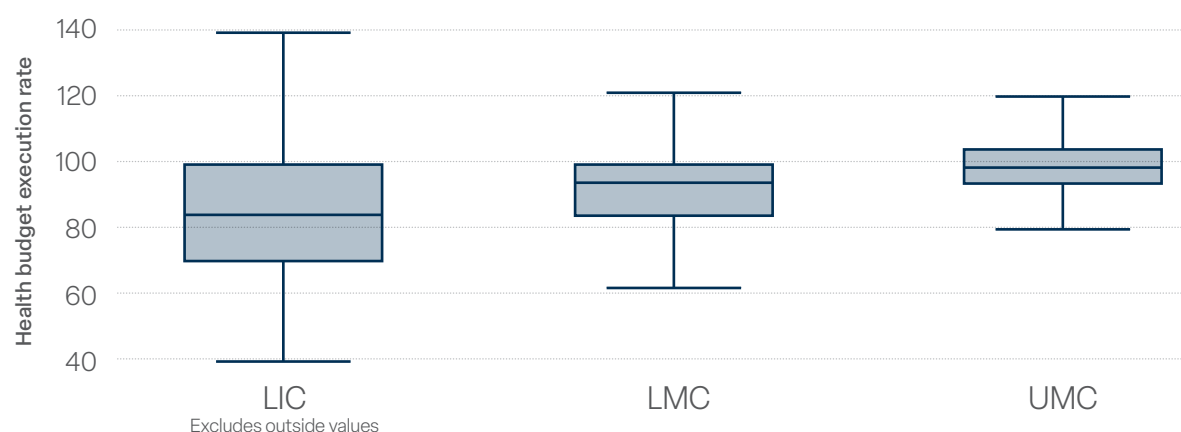
3.3 Overspending of health budgets is of concern, particularly for UMICs

The distribution of health budget execution rates indicates that countries across all income categories faced challenges both

with underspending and overspending. Figure 3.4 shows the extent to which health budget execution rates varied, across countries by income group, as box plots. The central portion of data for low-income countries spread over 30 percentage points from 69 percent to 99 percent, compared to 11 percentage points from 93 percent to 104 percent for upper-middle-income countries. The LICs' wider range reflects the greater variability in strength of their PFM arrangements; it also suggests

Figure 3.4

Health budget execution rates were spread over a wide range, indicating both under- and overspending (2010-2020)



Note: The box plots show the distribution or spread of the data where data are first ordered in increasing value and then divided into four equal parts or quartiles. The first quartile (values up to the 25th percentile) is represented by the bottom whisker to the bottom of the box. The second quartile (values between the 25th and the 50th percentile) is represented by the bottom of the box to the red line, which is the median value (50th percentile). The interquartile range (IQR) is the difference between the value at the 75th percentile and the value at the 25th percentile and is a measure of the dispersion of those data. The bars at the end of the top and bottom whiskers indicate the values which up to are 1.5 times the IQR; outliers lie beyond these whiskers.

their greater potential to improve budget execution. A country can address overspending by putting in place stronger oversight mechanisms – as compared to addressing underspending, which may require streamlining the release of funds.

3.4 Health budget execution rates in LICs were characterized by high year-on-year volatility

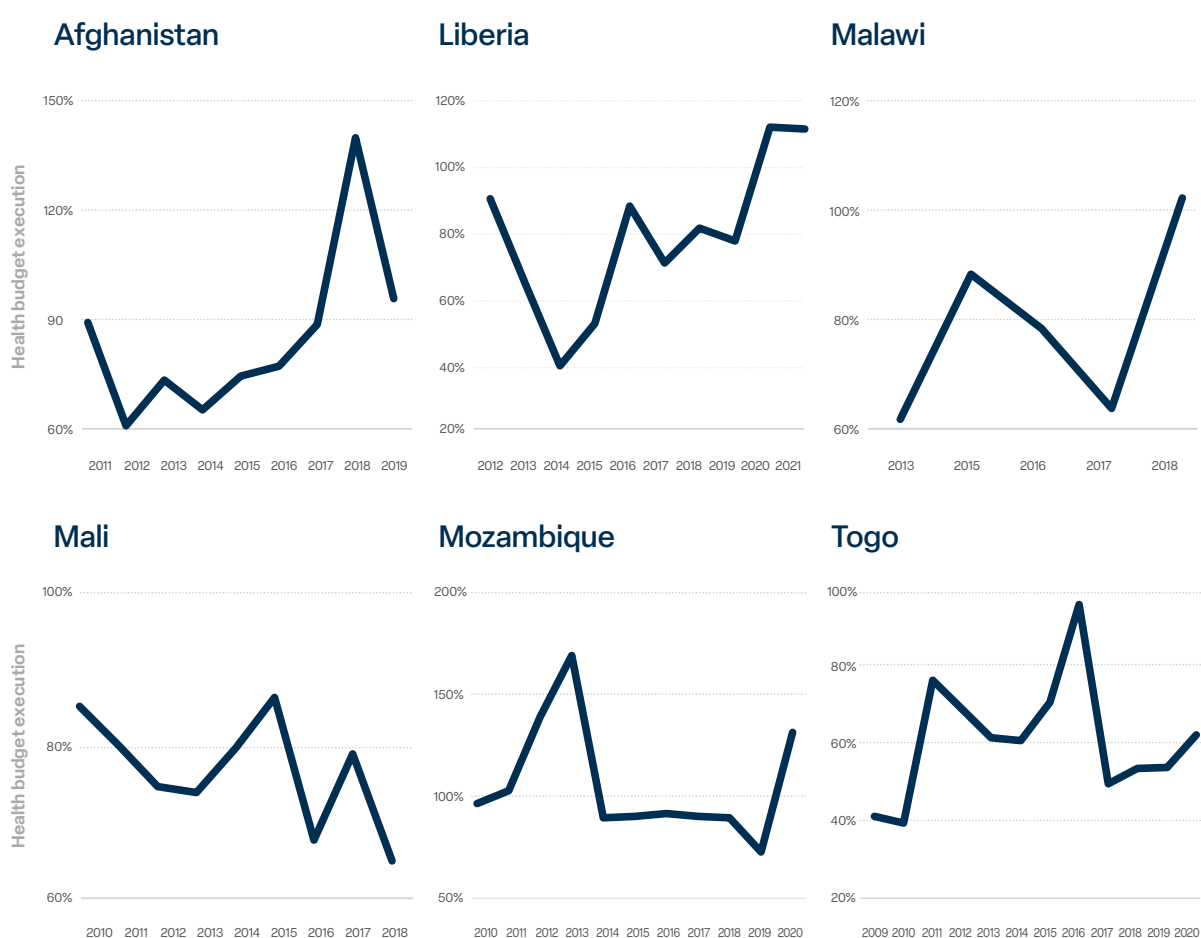
Not only were LICs' execution rates highly dispersed but year-over-year rate fluctuations were greater (Figure 3.2a and 3.2b). Figure 3.5 shows health budget execution rates for a selection of distinctive low-income countries (i.e., the ones with the most fluctuation) over time, presenting a pronounced rise-and-fall pattern. For example, Afghanistan's rates over the period 2011-2019 ranged from 65 percent to 142 percent with high year-over-year fluctuations, such as a drop from 90 percent in 2011 to 61 percent in 2012. Liberia's rates ranged from 40 percent to 112 percent over the period 2012-2021, with notable fluctuations from 52 percent in 2015

to 88 percent in 2016, and 71 percent in 2017.⁵ Year-over-year health budget execution rate volatility negatively impacts the sector's ability

to plan and budget, erodes budget predictability, and makes it difficult to ensure continuous health service delivery.

Figure 3.5

Health budget execution rates in selected low-income countries



⁵ In donor-dependent countries, development partners can increase the fluctuation of budget execution rates by channelling their contributions through the recipient-government's budget without also aligning their disbursements with the government's fiscal year (Piatti-Fünfkirchen et al. 2021a).

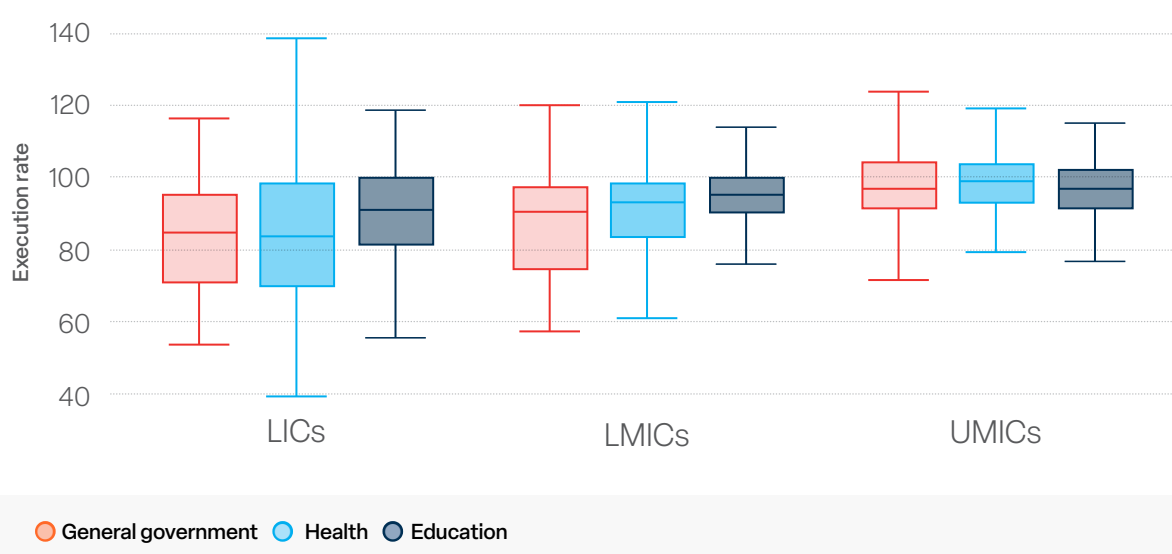
3.5 In the low-income and lower-middle-income countries, health budget execution rates were lower and had greater variability than in the education sector

When compared to the education sector, health budget execution rates tended to be lower and more highly dispersed for low-income and lower-middle-income countries. Figure 3.6 shows that over the 2010–2020 period, the low-income countries' median execution of their health budgets (in

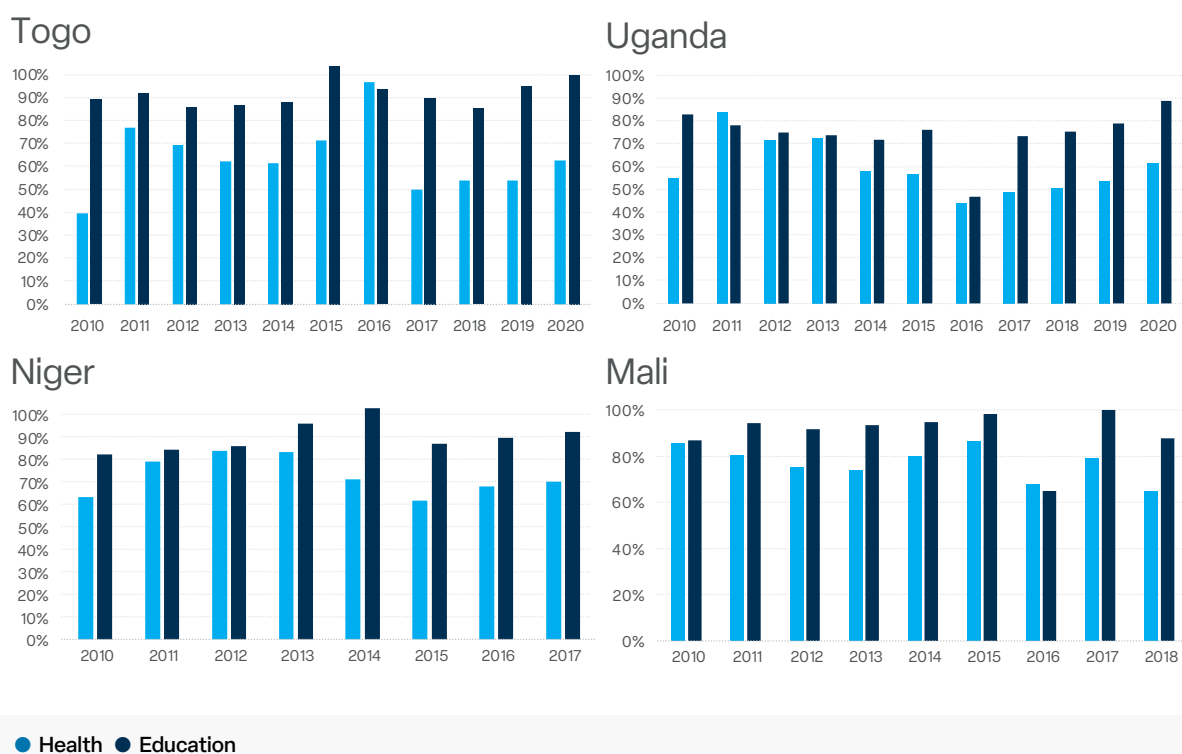
red) was 7 percentage points lower than for their education budgets (in cyan) (84 percent versus 91 percent). Differences are more pronounced in low-income countries. In low-income countries, rates for the health sector were also more dispersed, ranging from 40 percent to 139 percent compared to 56 percent to 119 percent in the education sector.⁶ In the upper-middle-income countries, by contrast, median budget execution for health was 2 percentage-points higher than for education (99 percent versus 97 percent), with a similar spread in the range of budget execution rates for both social sectors. The low-income countries' median health budget execution rates (84 percent) were similar to those for the government's general budget (85 percent). The upper-middle-income countries' median rates for health (99 percent) were also similar to those for the general government (97 percent).

Figure 3.6

Health budget execution was lower and more variable than education budget execution (2010–2020)



⁶ Excluding outliers.

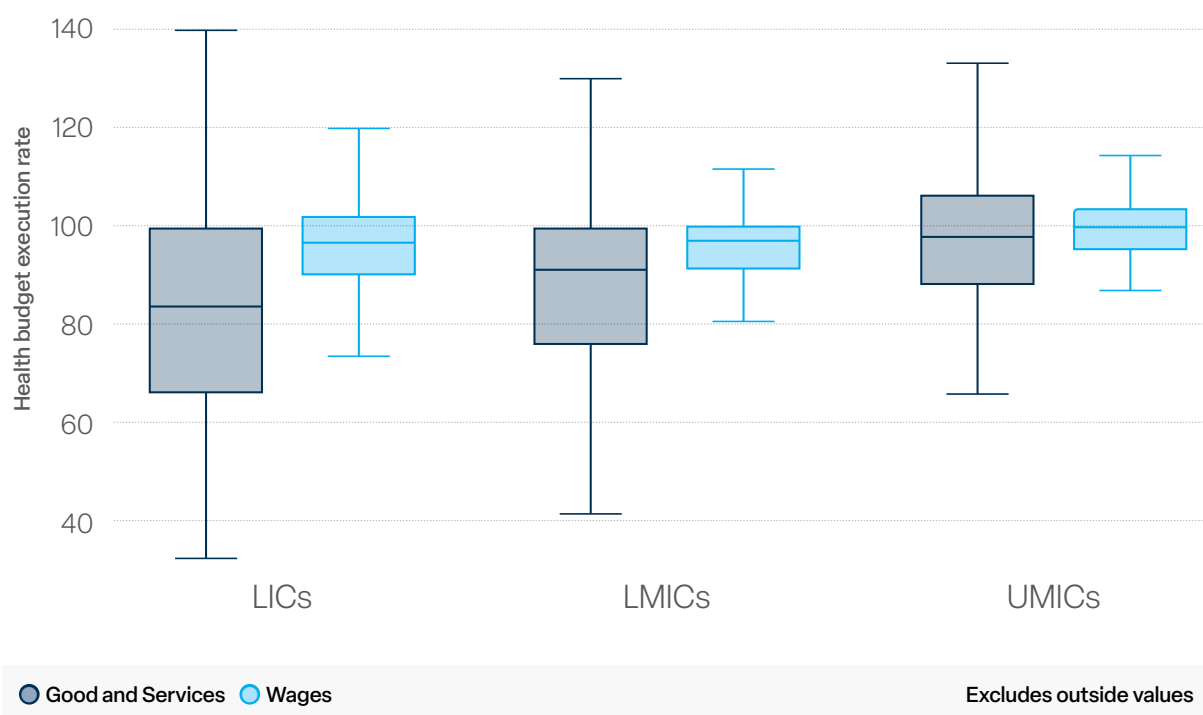
Figure 3.7**Health and education budget execution rates in select low-income countries**

3.6 All income groups underspent on goods and services in the health budget

Wages were often nearly fully executed while the portion of the health budget allocated to goods and services was much more likely to be underspent, in all three country income groups (Figure 3.8). LIC median wage execution rates were 97 percent compared to 84 percent for goods

and services. Execution rates for health goods and services were also highly dispersed in all three country-income groups as shown by longer length of the mid-spread box (IQR); in the low-income countries, the difference was 33 percentage points, between full execution, to execution of around just two-thirds of the approved budget (67 percent). Variation in spending on wages was also less in all country income groups; the largest of these was in the LICs' with a difference of 12 percentage points from 103 percent to 91 percent. The health sector's weaker budget execution for goods and services is a critical issue as service delivery requires not only

Figure 3.8
Health goods and services were underspent



effective implementation of budgets for wages but also for goods, such as drugs, masks, gloves, or other supplies.

‘This section has shown trends and distributional patterns in health budget execution rates across time and country income groupings. Findings highlight that closer analysis of specific countries’ challenges can provide further insight into root causes of suboptimal health budget execution performance and inform identification of potential solutions. The next section examines underlying health budget execution performance in 12 countries with the aim

of identifying common root causes of sub-optimal health budget execution and key drivers of better performance across stages of the budget cycle. Such information can help to identify potential solutions to unblock and fully use all available funds for the health sector.



4.0

COMMON CHALLENGES IN HEALTH BUDGET EXECUTION IN LOW- AND MIDDLE-INCOME COUNTRIES

This section identifies common health budget execution challenges in low- and middle-income countries, drawing from a scholarly literature review conducted from 2019-2020 and from 12 country case studies carried out jointly by WHO and the World Bank during 2021-2022.⁷

The purpose of utilizing these two sources was to gain a comprehensive understanding of the most common issues in health budget execution and the factors contributing to poor execution. This insight

aims to assist health and finance authorities in LMICs in their reform-making processes.

Literature review and case study findings are presented together, following the budget cycle, with the assumption that each stage impacts budget execution performance. A distinction is made between factors related to finance (or the Ministry of Finance) and those related to health (or the Ministry of Health). Country case study summaries are provided in Appendix 3.

4.1 Budget formulation

Figure 4.1

How problems in budget formulation can lead to challenges in budget execution in health

Challenge	Credibility of budget	Budget structure and rules for budget allocation
 Challenge	Inaccurate revenue projections	Slow transition to flexible budget structure
	Short timeframe to develop budget	Inadequate guidance
	Limited communication of budget ceilings to health	Limitations on budget re-allocations
 Ministry of Finance	Overuse of historical budgeting	Continued use of input-based budgeting
	Input-based planning	Inappropriate application of MOF guidance
	Inaccurate cost estimation	No alignment between allocation and implementation responsibility
	Poor priority setting	
 Ministry of Health		

⁷ The country case studies conducted by the WHO and the World Bank in 2021-22 analyzed budget execution rates quantitatively, and used a consistent approach to explore key drivers both of positive and negative performance informed by the findings from the literature review. The 12 countries, selected to represent a diverse mix of geography, income level, population size, and institutional structures, were Burkina Faso, Cameroon, Democratic Republic of Congo, Ethiopia, Kyrgyz Republic, Lao People's Democratic Republic, Pakistan, Senegal, Solomon Islands, Timor-Leste, Uganda, and Ukraine. The case studies found that almost all countries (10/12) had aggregate health budget execution rates above 85percent, which on the surface suggested fairly good performance, but that the countries' disaggregated health budget execution rates and qualitative responses showed underlying issues impeding effective and efficient budget implementation. The case studies were specific to the countries featured and cannot be considered representative of all LICs' and LMICs' situations.

Budget credibility

Budgets often lack credibility, which creates challenges for execution. A credible budget is one that is prepared with realistic and achievable revenue projections and budget estimates. These characteristics increase the likelihood that the government will be able to raise revenues and to spend funds in line with the allocated budget (Allen and Tommasi 2001, Barroy et al. 2019, IBP 2019). Weak credibility of health-sector budgets can stem both from general government finance-related issues and from issues specific to the health sector.

General finance-related factors which can compromise budget credibility include inaccurate revenue projections, compressed budget timetables, and inadequate communication regarding budget ceilings.

These factors often result in unrealistic or error-prone budget estimates and revenue projections, making it challenging for the government to raise sufficient funds or to execute the budget as planned. The government may then be required to make in-year adjustments, such as virements or supplementary estimates (Cashin et al. 2017, Grinyer 2019, Ally and Piatti-Fünfkirchen 2021, Schiavo-Campo 2017, Pattanayak 2016).

Health-sector-specific factors, such as unrealistic cost estimates, the persistence of historical budgeting practices, and reliance on input-based planning, can also weaken budget credibility. When health authorities' cost estimates are unrealistic, this can give rise to substantial gaps in financing, with costs that

exceed appropriated allocations. Input-based planning that does not align with current needs can hinder health program implementation. These factors can be compounded by a lack of technical capacity in budget development within the health sector and by allocating insufficient time for budget preparation (Barroy et al. 2019).

Credibility also refers to the alignment of budget allocations with stated priorities and objectives. Budget credibility suffers when finance and health authorities fail to align budget with health needs during the budget-formulation process. From the health sector's perspective, such a lack of alignment can result in a mismatch between the allocated resources and the sector's financing needs during the budget execution stage (Barroy et al. 2019).

In most case study countries⁸ the health sector's budget was not considered credible, creating budget execution issues downstream. Poor quality cost estimates led to overruns or underruns in health expenditure (for example, in Pakistan), and challenges in delivering health services when **the allocated amount was insufficient for operational needs** (for example, in the Kyrgyz Republic). In other instances, **inaccurate revenue projections often led to incomplete or delayed fund releases**, sometimes resulting in unpredictable funding for budget implementation (for example, in the Lao People's Democratic Republic).

Strategic planning was generally focused on longer-term sector plans, which in turn was not well integrated into the budgeting

8 In the rest of this section, "countries" refers to the 12 countries reviewed.

processes. While governments invested significant time in developing cost estimates for these long-term plans, those estimates did not always align with the budget allocation process and timeline, and therefore considered a binding constraint of projected resources.

Many of the countries prepared strategic plans that had large financing gaps between estimated costs and expected resources, such as Senegal's gap of around 70–90 percent. Several of the countries, including Burkina Faso, Cameroon, and the Kyrgyz Republic, were not using their Medium-Term Expenditure Frameworks (MTEFs), an approach to budgeting which can address annual budgeting shortcomings (World Bank 2013), to inform their annual budget allocations.

Annual budgeting processes tended to rely more on historical estimates rather than on costing estimates to determine budget allocations. The reasons given were insufficient time and capacity (for example, in Burkina Faso, the Kyrgyz Republic, Pakistan, the Solomon Islands, and Timor-Leste). This tendency persisted despite the knowledge that inaccuracies in estimates had previously led to budget execution challenges. In the Democratic Republic of Congo, for example, there were many instances where the same line-item budget allocations were given year after year even though most of these allocations did not end up being executed, even as other line items were consistently over-executed. Further, unit costs need to be updated to reflect actual costs of line items. If unit costs are inadequate in the budget, audit irregularities may arise. It may then be safer not to spend to avoid being held accountable.

Budget preparation processes were often centralized and excluded lower-level health facilities, sometimes contributing to cost

estimates that were unrealistic, as was the case in Pakistan, Timor-Leste, and Uganda.

The extent of centralization was affected by the balance between a country's Ministry of Finance and its Ministry of Health, as well as between the central government and provincial or regional entities. A lack of input from health service providers sometimes contributed to a mismatch between what was allocated and what was needed for service delivery. Mismatches also resulted at times when governments cut budget lines to fit within tighter fiscal constraints.

Robust revenue estimates were complicated by unpredictable fiscal contexts and uncertainties regarding donor funding.

Fiscal challenges in many of the case-study countries contributed to shortfalls in funding compared to original budget allocations (for example, in the Kyrgyz Republic, Pakistan, and Uganda). Countries where the health sector is reliant on external donor support sometimes experienced challenges receiving accurate estimates in the right format, for the right time-period, and in time to inform their budget process (for example, in the Democratic Republic of Congo and the Solomon Islands).

Budget structure and rules for budget allocation

Spending on health, more so than spending in most other areas, requires flexibility to adjust allocations to evolving needs in order to enable full budget execution (Barroy et al. 2019, Piatti-Fünfkirchen, Barroy et al. 2021b). However, such flexibility is often lacking or underutilized in LICs and LMICs. PFM-related regulations and rigid budget structures such as

line-item budgeting can reduce the ability to reallocate funds across budget lines (Barroy et al. 2018). Even in countries which have put in place more flexible approaches, such as program-based budgeting (PBB), the desired flexibility may not always be available in practice. Regarding PBB specifically, impediments to the necessary flexibility may include incomplete adoption of PBB or a lack of guidelines to facilitate health sector PBB implementation. Even when PBB is in place, many LICs and LMICs continue to use input-based or line-item budget structures when developing health budgets, increasing the likelihood of budget under-execution. (Barroy et al. 2018, Piatti-Fünfkirchen, Hashim, and Farooq 2019, Barroy, Blecher, and Lakin 2022).

Most of the case-study countries had significant budget rigidities. Budget flexibility was identified as particularly critical in the health sector, given that health service needs were often demand-led and influenced by external factors that cannot be known in advance, and therefore were difficult to predict in the budget preparation process. Most countries relied on input-based budgeting processes, which prioritize control over flexible resource use. In certain countries such as the Democratic Republic of Congo and Uganda, public finance laws required parliamentary approval for relatively minor readjustments. In countries such as Ethiopia and the Solomon Islands which had lower legal requirements, time-consuming and challenging bureaucratic processes were sometimes still required. In Burkina Faso, for example, reallocating resources during budget execution meant following cumbersome steps. In the Kyrgyz Republic, the inability for the health

ministry to rapidly reallocate resources contributed to both lower budget execution rates and operational inefficiency. Although controls are foundationally important, countries must identify when to allow greater flexibility when reallocations are required. Allowing some flexibility to reallocate budget allocations in response to changing needs is considered essential to support effective budget execution in health.

Program-based budgeting has the potential to aid budget execution by increasing flexibility but was only partially applied in most of the case-study countries.

Even countries such as Burkina Faso, the Kyrgyz Republic, and Uganda - which had introduced program-based budget structures - continued to use input-based budgeting principles to manage execution processes. In Ukraine, by contrast, managers of each budgetary program had the autonomy to reallocate resources within the fiscal year, with a view to delivering the outputs identified in the original budget. In general, there was little evidence showing better budget execution after program-based budgeting reform introduction.

4.2 Budget execution

Figure 4.2

Key challenges during budget execution phase

Budget execution

 Challenge	Regularity and predictability of funding flows	Appropriateness, and effectiveness of spending protocols	Effectiveness and efficiency of procurement and capital investment operations
	Delays and incomplete budget releases	Cumbersome authorization and spending procedures	Multi-layered and over-centralized procurement rules
 Ministry of Finance	Cash rationing	Excessive centralized control (no delegation of authority)	Lack of appropriate procurement protocols
	Health budget deprioritised in-year		
 Ministry of Health	Delays in cash requests from health	Lack of autonomy for service providers	Constrained ability to develop and implement procurement plans
	Fragmented health financing flows	Fragmented spending rules by financing source	Inefficient operationalisation of capital investment
		Limited financial management capacity	

Regularity and predictability of funding flows

Weak cash management practices are common in LMICs, making funding availability unpredictable (Simson and Welham 2014, Ally and Piatti-Fünfkirchen 2021). Regular and predictable fund release enables timely payments, timely implementation, and close adherence to budgetary allocations. Key factors that contribute to effective fund release are

effective cash management practices such as revenue collection in line with estimates, careful cash flow monitoring, and proper management of commitments and payments (Schiavo-Campo 2017, Ally and Piatti-Fünfkirchen 2021, Barroy et al. 2019).

In the absence of strong cash management practices, countries often experience downstream execution challenges such as incomplete disbursements, missed or

delayed payments, build-up of arrears, and end-of-year expenditure sprees (Pattanayak 2016, Potter and Diamond 1999). Due to revenue shortfalls, many countries have resorted to cash rationing practices, where spending is based on cash availability rather than on adhering to the budget (Schick 1998; Schiavo-Campo 2017). Such rationing practices compromise the predictability of funding and the capacity to fully implement the budget.

The release of funds can also be compromised due to mid-year deprioritization, where funding, particularly if constrained, is allocated elsewhere as priorities shift over the course of the year. This can often stem from poor health sector involvement during the in-year budgetary negotiation process, changes in political incentives, emergencies, and/or funding shortfalls (Barroy et al. 2019, PEFA 2016, PEFA 2020).

While the finance ministry is broadly responsible for the government's cash management, health-sector-related factors also contribute to challenges in the release of funds to the health sector. Health-sector actors can be delayed in submitting cash requests and expenditure plans, leading to irregular or incomplete fund release for health service delivery. **Additionally, the health sector's often complex and fragmented financing landscape can make it challenging to manage and disburse funds on time and in full, given that each individual funding flow may have different requirements for its release** (Barroy et al. 2019, Rajan, Barroy, and Stenberg 2016).

Unpredictable fund release further complicated budget execution. In many case-study countries, fund transfers were often delayed, incomplete, or didn't occur altogether. In Pakistan, for example, facilities reported that the unpredictable release of funds had harmed service quality and **compromised their ability to meet critical operational costs such as utility bills.** In Ethiopia, by contrast, the finance ministry adhered to the budget calendar, and communicated the budgets for upcoming fiscal years, including details of transfers to subnational levels, to regions and woredas⁹ in time. These good practices helped the regions and woredas with their planning and commitment processes and supported high-quality budget execution.

Weak cashflow management was another impediment to effective budget execution. Effective cash management requires accurate revenue forecasts, oversight of commitments, knowledge of upcoming payments, and robust cash plans to ensure cash availability at payment due dates. Cash management can be particularly complex in the health sector, especially when financing sources are highly fragmented, as was the case in Ethiopia, the Solomon Islands, and Uganda. In addition, as was the case of Pakistan, poor-quality revenue forecasts contributed to challenges in managing cashflow which sometimes resulted in the budget being under-executed.

Several case studies reported the use of cash rationing to manage funding flows. **When limited cash is available, countries may resort to cash rationing, making decisions**

⁹ Woredas are the third level of administrative divisions, after zones and regional states (equivalent to districts in many other countries).

based on cash availability rather than on budget plans. This type of system is unpredictable and makes smooth budget implementation difficult. Timor-Leste, for example, systematically delayed its budget transfers during the first part of the fiscal year. In the Kyrgyz Republic, cash rationing practices led to withholding 30 percent of the budget for the first ten months of the budget year for non-protected items, including capital and non-salary expenditures. Based on cash availability, the government released the remaining budget allocations in the final two months of the year, creating a surge in funding with limited time to implement the budget as planned.

Appropriateness and effectiveness of spending controls

Effective spending controls are essential for ensuring compliance, timely payments, and successful activity implementation.

Striking a balance between control and flexibility is essential, however.

An excessive number of ex-ante controls can create implementation bottlenecks without necessarily improving compliance. In many LMICs, the spending controls set by finance authorities have been criticized for being cumbersome and too centralized, contributing to reduce the ability to fully and effectively spend (Cashin et al. 2017; Barroy et al. 2019, Pattanayak 2016). Procurement-related controls tend to be particularly cumbersome and are often referenced as one of the main causes of poor health budget execution (UNICEF 2020, Pattanayak 2016).

Health sector factors that impede effective health budget execution include limited financial management capacity and constraints on service-provider autonomy within a complex and fragmented financing landscape.

The health sector's complex and fragmented financing ecosystem results in an array of spending rules and controls which vary by funding source, which can lead to inefficiencies and errors when executing the budget (Barroy 2019; McIntyre et al. 2008). In addition, financial management capacity is often at its weakest in first-contact health facilities, often resulting in inefficiencies and errors in executing controls. Often, given their lack of autonomy, these facilities do not manage their allocated resources directly. In turn, the remote management of those resources by districts/regions contributes to drag and inefficiencies in managing transactions (Barroy, Blecher, and Lakin 2022, Piatti-Fünfkirchen and Schneider 2018; WHO 2022).

In most of the case study countries, spending controls tended to be inefficient, overly bureaucratic, and time-consuming.

Excessive controls can result in weak execution rates, payment processing delays, and activity implementation interruptions. Respondents highlighted that the approval and authorization of transactions tended to be multilayered and excessive, and that the processes to release cash and payments could be equally extensive and require complex payment systems. In Timor-Leste, for example, health facilities had to secure approval at multiple levels before conducting transactions. As part of the process, they were required to input commitments in two systems with different procedures and requirements. The operational inefficiencies that resulted impeded budget execution.

Several case studies noted the use of stringent procedures even for payments of low value. In Pakistan a bill of 200 rupees was subject to the same rigorous controls as one worth over a million rupees. The payment system also had duplicate internal controls, requiring authorization and approval from several officers from different entities and making the process cumbersome and bureaucratic.

An over-centralization of controls sometimes created execution inefficiencies. In countries such as Burkina Faso, Pakistan, Senegal, and Timor-Leste, health systems often relied on higher levels of government to execute authorizations, commitments, and payments. While this centralized approach occasionally fostered effective aggregate control and compliance with the approved budget, it also sometimes made day-to-day operations difficult for health facilities, and created mismatches between approved transactions and needs. In the Solomon Islands, the central-level Ministry of Health was responsible for managing most transactions such as processing payments even though it was the provinces which implemented most health activities. Stakeholders commented that this centralized system to authorize transactions led to budget execution delays.

Health facility lack of autonomy to manage their resources was a major cause of budget execution inefficiencies. This challenge was noted in most case study countries (for example, Pakistan, the Solomon Islands, and Timor-Leste). Certain countries granted autonomy to their health facilities while still constraining them, such as the Kyrgyz Republic which required facilities to obtain approvals from higher levels before reallocating resources

within their budgets. By contrast, countries where health facilities had autonomy, such as Lao PDR and Ukraine, recorded execution rates of close to 100 percent. Ukraine's flexible direct transfers to facilities improved process execution efficiency, admittedly with a loss of transparency in resource utilization.

Payroll control weaknesses caused serious problems in budget execution in certain countries, even where execution rates were high. While most of the countries recorded high rates of execution for salaries and wages, these were not without problems. The Democratic Republic of Congo and the Kyrgyz Republic had cases of ghost workers (that is, payments going to individuals who did not work for any health facility), linked to weaknesses in payroll controls. By contrast, Ethiopia's payroll controls included nearly real-time updating of the payroll for units within the IFMIS, and within weeks for units outside the IFMIS; and Uganda's automated payroll system contributed to more reliable wage-bill estimates and better execution.

Procurement effectiveness and efficiency and capital investment operations

Many LMICs struggle with inadequate public procurement systems characterized by excessive centralized administrative controls, and cumbersome procedures that are not specifically tailored to health sector needs.

Additionally, the health sector's weak technical capacity impedes its ability to take steps such as realistic operational plan formulation, tenders, and contracts). Given that governments channel a significant proportion of their health expenditure through the public procurement system, these

procurement inefficiencies are widely recognized as one of the main causes of poor budget execution in health. Capital projects within the health sector commonly experience similar challenges, with inadequate technical capacity and limited expertise in project management resulting in delays or suboptimal utilization of allocated funds (UNICEF 2020, Pattanayak 2016).

Procurement processes were often complex, lengthy, and bureaucratic, especially for health products. For example, in Ethiopia, drug and medical commodity procurement was found to require a year on average to receive the internal and external approvals. Similarly, in Timor-Leste,

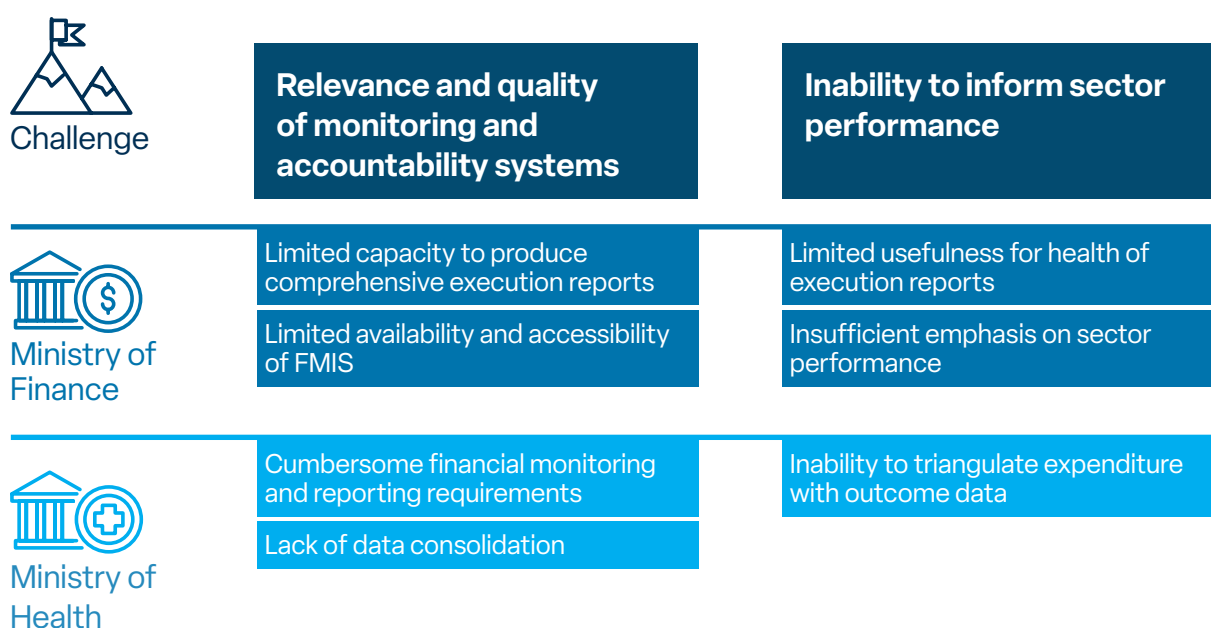
it was reported that payments made through the procurement process could take several months to be released due to the number of approvals required. In Burkina Faso and Senegal, procurement plans were late and incomplete, and only a limited range of suppliers could meet the specific standards set under the centralized procurement process. In the Kyrgyz Republic, a lack of capacity among procurement staff to produce technical specifications and tender documents led to procurement rules not being followed, incorrect documentation being submitted for approval, submissions being made with errors, and suppliers submitting supporting documents late and of low quality.

4.3 Budget oversight

Figure 4.3

How budget oversight challenges contribute to suboptimal budget execution in health

Budget oversight



Relevance and quality of monitoring, and accountability systems

Many countries fail to collect adequate data to effectively inform quality budget oversight. Budget oversight involves collecting information to monitor and control spending, to ensure funds are used according to their intended purpose, and to assess how well the government has executed the budget (Dorotinsky and Watkins 2013).

LMICs' limited capacity to produce comprehensive execution reports commonly constrains effective budget oversight.

The combination of inadequate financial management information systems (FMIS) and limited capacity in financial management capacity compromises governments' ability to ensure quality budget oversight. Budget oversight in the health sector is made more complex by fragmented funding flows, which require multiple reporting mechanisms, including some that operate outside the FMIS framework. This specificity makes it harder to develop a comprehensive and consolidated picture of the sector's spending (Barroy et al. 2019; Piatti-Fünfkirchen, Barroy et al. 2021b; ThinkWell and WHO 2022b).

The ability to monitor budget execution in real time depended on the quality and timeliness of financial reporting. In Cameroon, for example, there was no functional automated system for monitoring budget execution. In other countries such as Burkina Faso, Ethiopia, Pakistan, and Timor-Leste, automated reporting systems were in place, but connectivity challenges led to data reporting

delays, particularly from decentralized authorities and facilities. In the Kyrgyz Republic, capacity issues at lower levels sometimes led to inaccuracies in data. Certain countries such as Ethiopia relied on multiple systems to monitor execution performance given their multiple financing mechanisms. The Kyrgyz Republic fostered better monitoring of its formerly fragmented resource flows by introducing reforms in 2018 to ensure that financing for contracted health care organizations (HCOs) was transmitted through a single channel, a change that improved oversight.

Audit processes sometimes influenced budget execution effectiveness. External audits tend to be conducted after the execution process, but their nature also influences actions during spending processes. In Ethiopia, for example, fear of punishment by auditors prevented budget owners from fully implementing their budgets. In Ukraine, audits had become increasingly punitive, with the result that they “demotivate[d] providers to exercise their new managerial flexibility and increase spending efficiency.” In the Kyrgyz Republic, managers steered away from a more “proactive and autonomous management” of budgets due to fears of excessive and difficult audits. Given that most of the countries' budgets were effectively input-based, audits tended to focus on compliance rather than on output delivery. There was little mention of internal audits, which are typically conducted while execution processes are ongoing and can help in addressing issues while there is still an opportunity to avoid delays.



Inability to inform sector performance

The tendency to focus more on input-based accountability rather than output- or performance-based accountability

perpetuates a system that prioritizes compliance over evaluating whether the budget has effectively realized its intended outcomes. The health ministry also commonly lacks the necessary data, and/or expertise, to be able to triangulate spending indicators with operational outputs. This makes it challenging to understand how well the money spent correlates with performance. Addressing these challenges is crucial for improving how governments oversee and manage their budgets, and thereby to strengthen their budget execution.

The literature from LMICs and the case studies of 12 LICs and LMICs both show that health budget execution was directly influenced by the other phases of the budget cycle and by the actions of multiple actors. How countries executed their health budgets depended on their spending capacities and processes (the

budget execution phase), as well on how they allocated and formulated the budget (budget formulation stage) and how they accounted for and reported on the budget (budget oversight stage). A key learning agenda emerging from this analysis is that focusing only on expenditure management (the execution phase) is too narrow; improving the performance of health budget execution involves strengthening and tailoring the budget formulation and oversight functions as well.

Budget execution performance is the result of interventions by multiple government actors – principally in finance and health, with local authorities also being crucial in decentralized contexts. Country evidence demonstrated that good health budget performance was not only the result of better absorption of budgeted resources from the health ministry; actions are also necessary from the finance side to make PFM systems more agile and responsive to health needs to enable full budget execution. The next section examines possible policy actions that countries might introduce or accelerate, to address the most commonly observed health budget execution bottlenecks.

The background is a solid dark blue with a complex pattern of faint, light blue icons and lines. These include a large upward-pointing arrow in the top left, a network of dots and lines, a line graph with multiple peaks and valleys, a bar chart, a hexagon containing a person icon, a hexagon with a plus sign, a hexagon with a bandage, and various other geometric shapes and symbols. The overall aesthetic is modern and technical, suggesting themes of technology, healthcare, and business.

5.0

OPTIONS FOR POLICY ACTIONS AND DIALOGUE

This section explores what LMIC governments can do to address budget execution challenges in health.

We identify opportunities for engagement by finance, health, and local government authorities in limited-resource contexts. These opportunities are grouped in line with the analytical framework presented in previous sections:

- i. budget credibility;
- ii. budget structure and rules for budget allocation;
- iii. fund flow regularity and predictability;

iv. spending control appropriateness and effectiveness;

v. procurement and contract management; and

vi. monitoring and accountability systems.

In each of these areas, we present the opportunities and potential reform actions by stakeholder, illustrating these with examples from the twelve studied LMIC cases.

For each of these areas, we offer options that can be tailored to individual country contexts.

5.1 Strengthening health budget credibility

Table 5.1

Potential actions for finance and health ministries, and local governments

Ministry of finance actions	Ministry of health actions	Local government actions
Strengthen capacity for accurate revenue projections	Inform MOF when budget ceilings are inaccurate	Advocate for reliable fiscal transfers, as relevant
Protect integrity of projections against political pressures	Create pressure for improved budget ceilings by identifying links between revenue performance and service delivery outputs	Set up a process for credible and reliable development of sectoral budgets
Accelerate multiyear budgeting reforms	Strengthen technical capacity to develop more accurate cost estimates	Clarify roles and responsibilities for budget development (for example, between district and health facilities)
Allow sufficient time for budget preparation	Strengthen coordination between health planning and budgeting/finance division in the MoH	
Communicate budget ceilings to sectoral ministries in a timely fashion	Set up a process for within-year prioritization, in case of shortfalls	

What can a ministry of finance do?

Public finance theory presents numerous options, both on the revenue side and the expenditure side, to address deficiencies in the credibility of a country's general budget.

On the revenue side, investments in technical capacity, via support for financial and statistical data and modelling, will enable the preparation of more credible revenue projections. In turn, more credible projections will foster more realistic expectations of the available resource envelope for all sectors, including health. Equally important is an enabling environment that protects modelling from pressures to develop overly optimistic projections. On the expenditure side, realistic costing and budget planning are fundamental to developing a budget that corresponds to actual expenditure needs. Adopting and implementing medium-term fiscal and expenditure frameworks also can contribute to more credible budgeting. The ministry should communicate a realistic budget ceiling to individual sectors, including health, and allow those sectors sufficient time to consult and prepare their budgets. In many countries, strengthening these upstream PFM processes is part of ongoing reforms, across all sectors including health (World Bank 1998, IMF 2013, Schiavo-Campo 2017).¹⁰

What can a health ministry do?

The MOH may not be able to affect core finance rules, practices, and operations, but it can coordinate and communicate more effectively with the MOF, and can advocate the MOF for improvements.¹¹ For example,

the MOH should communicate to the MOF if health budget execution and service delivery are impeded by inaccurate budget ceilings or late and inadequate cash releases. Dialogue with the MOF can also help clarify whether problems in budget execution are an issue of absorption-capacity within the MOH or related to the broader PFM environment.¹² The MOH can improve the credibility of its budget proposals by first investing in internal capacity to estimate its proposed costs more accurately. More accurate proposals will, in turn, give the MOF confidence that the amounts allocated will be fully executed. Additionally, the MOH could anticipate and respond to upstream PFM issues more deliberately by setting up a process to identify any shortfalls in budget allocations or cash-rationing needs earlier, and, as a second-best option, to discuss within the sector how to apportion funds to priority areas during a given fiscal year.

10 In environments where upstream PFM problems repeatedly affect the credibility of health service providers' budgets, the finance authority can take measures to allow providers to react appropriately. In some countries providers collect user-fees to protect against budget shortfalls and cover the cost of services. While the collection of user fees is not encouraged due to equity and access concerns, in instances where facilities are collecting such fees it would be advisable to allow the facilities to retain those fees to cover any cash shortfalls. Sweeping those fees back to treasury is likely to have unintended consequences such as underreporting of user fees, creating challenges for accountability.

11 One such step could be working with the MOF to set up a joint platform for frequent dialogue relating to health service delivery needs and financing; this could help in anticipating financing shortfalls and meeting peak demand.

12 If it turns out that budget execution problems result from the broader PFM environment, the MOH should both document this so it does not affect the MOH's ability to advocate for domestic budget allocations, and clearly communicate to the MOF the impacts on health service delivery and population effects, to encourage the MOF to address any shortcomings.

What can a local government do?

Many of the issues raised above also hold true for local government institutions, on both the health and finance sides, as do the need for communication, collaboration, and prioritization. In addition, in countries where intergovernmental transfers make up a large share of local governments' revenue, making these transfers reliable is important as this affects how credible local government budgets can be and, in turn, how well they will be able to execute those budgets. Setting up a process

that prioritizes these transfers and treating them as quasi-statutory payments (similar to salary payments, for example) is critical, especially in contexts where local governments pay for the cost of local service delivery directly. Local governments should also clarify the respective roles and responsibilities of local government and of health service providers. By establishing a dedicated budget for service providers for example, the local government could help service providers to access the full allocated budget and be better equipped to execute it.

Box 5.1:

Strengthening budget credibility in Ethiopia

Most countries are working on broader PFM reform that affects credibility of the general budget. Ethiopia is an example of a country which had certain components supporting budget credibility in place but needed to strengthen others. The central-level government maintained a realistic budget calendar, and clearly communicated the details of transfers to subnational levels. This approach helped local government units with their planning and budgeting processes, key factors in developing more credible budgets. Nevertheless, health facility budgets were often inflated, and, when senior officials detected this, they reduced the amounts transferred to those facilities. As a result, Ethiopia continued to struggle with producing credible estimates.

5.2 More appropriate budget structure and improved rules for budget allocation

Table 5.2
Potential actions for finance and health ministries, and local governments

Ministry of finance actions	Ministry of health actions	Local government actions
Accelerate delegation of spending authority to and within the MOH, where not yet complete	Work closely with MOF to identify challenges in the current budget structure and opportunities for reform	Same as for central level
Provide guidance to MOH on budget formulation reform	Provide sector-specific guidance on budget structure reforms including PBB design	Opt for consistent PBB structure at subnational level
Coordinate with MOH on design of health-specific budgetary programs, as part of adoption of PBB	Avoid disease-oriented programs in PBB structure	Align budget structure with health facilities' service delivery mandate and arrangements
Ensure that PBB, once introduced, is adopted fully	Advocate to the MOF to incorporate major inputs into budgetary envelopes, to enable full execution and efficient spending	Delegate spending authority to health service providers
Align disbursement rules to PBB principles		

What can a ministry of finance do??

Addressing rigidities in “budget holding” is a priority when aiming to streamline budget execution. Often, health service providers (generally at primary care level) do not have the status of spending unit and therefore cannot define, receive, or manage budgets. Recognizing service providers in the chart of accounts or, at minimum, allowing them to receive budget resources, is a critical first step toward optimizing

budget execution in health. In addition, a finance ministry should ensure that the budget structure is sufficiently flexible. Reforms to revise budget formulation and the related rules for budget allocations are complex processes that require detailed guidance from finance authorities. In a country which introduces a PBB structure, the new structure can add a layer of controls and complicate execution of the budget if the MOF does not take certain additional steps. The MOF and sectoral ministries including the MOH should

work together to define budgetary programs and implementation arrangements. Gradually implementing, refining and addressing specific problems associated with the budget structure is likely to help the MOH implement its budget well. To ensure sufficient flexibility in budget disbursements with PBB, the MOF should remove line-item controls. This can be an effective mechanism to improve budget allocation execution. (Barroy, Blecher, and Lakin 2022, Brumby et al. 2022)¹³

What can a ministry of health do?

The MOH should set up a budget structure that serves the health sector’s needs and priority policies while following the general guidance provided by the MOF. If the country is transitioning to PBB, the MOH should strive to facilitate budget execution and to avoid financial fragmentation, in part by avoiding defining its PBB structure by vertical disease-oriented programs. In addition, the MOH should incrementally incorporate the main inputs of its operational programs (such as for maternal health, primary care, or access to care) into budgetary envelopes, rather than maintaining

separate budget lines and rules for itemized inputs such as staff, drugs, or non-capital equipment (Barroy, Blecher, and Lakin 2022). Aligning the budget structure with service-delivery arrangements can simplify budget-execution processes.

What can a local government do?

Budget structures are often disparate across subnational levels. In countries which have introduced PBB, subnational-level authorities should adopt a consistent PBB framework for all entities at the same level (e.g., a region, province) to streamline budget execution and enable consistent expenditure tracking. It can also facilitate budget execution by serving as a “pool” of resources (combining local revenues, transfers from the central-level government, and external resources, where relevant) channeled through a consolidated stream of funds to health service providers. In addition, the local government should recognize health facilities in their local budget’s structure, enabling those facilities to receive a budget, execute that budget, and be more directly accountable for the resulting outputs.



¹³ The MOF and the sectoral ministries, including health, can negotiate middle-ground solutions such as complementary reporting by programs and certain line items such as staff and capital, which can help balance flexibility and control. If this approach is chosen, the government would need to structure its FMIS by program and by economic items.

Box 5.2:

Reforming the budget structure in conjunction with streamlining spending protocols in health: the case of Burkina Faso

In Burkina Faso, the institutionalization of PBB improved the execution of the health budget. MoH defined a primary care budgetary program in the MOH's PBB structure and further amended rules for disbursements (through a MOH decree), as a means of streamlining spending. This new modality provides direct budget transfers of PBB resources to primary care providers, through district bank accounts, to cover operating costs. MOH performs ex-post controls based on outputs, to track performance and ensure transparency and accountability. The budgetary program's execution rates were reported to be satisfactory despite budget challenges in meeting increased demand for services. Where budget structure and disbursement rules are not aligned, and supporting systems not in place, PBB can however reveal weakness in budget execution processes (as observed e.g., in Gabon (Aboubacar et al 2020)).

5.3 Regularity and predictability of fund flows

Table 5.3

Potential actions for finance and health ministries, and local governments

Ministry of finance actions	Ministry of health actions	Local government actions
Strengthen general cash management practices	Strengthen the MOH's cash management practices, and submit credible cash requests on time	Strengthen local government's cash management practices
Recognize shortcomings in treasury single account operations and allow for exceptions where appropriate	Agree on priority budget lines on an ex-ante basis to be applied in the event of cash rationing	Advocate for predictable and adequate intergovernmental transfers
Make provisions to allow health facilities to retain internally-generated funds	Invest in financial management capacity across spending units	Coordinate funding and ensure priorities and objectives are aligned, not conflicting or duplicated
		Invest in local government's financial management capacity

What can finance ministries do?

Improving the management of cash is a typical function of central-level MOFs.

This activity involves strengthening capacity for adequate cash forecasting and finding ways to transfer funds efficiently and with certainty.

Working on treasury single account (TSA) reform can strengthen cash management, benefiting all sectoral ministries including health. However, many LMICs continue to experience considerable cash management problems and in the absence of a fully-functioning TSA it is helpful to consider temporary exemptions that can ensure the MOH has timely availability of cash to conduct its priority activities (delivering health services). Specifically, MOFs have two options available:

- i. recognizing the TSA's shortcomings, allow small cash balances to remain in dedicated accounts outside of the TSA (for example in health facility accounts), which the account-holders can then draw on as needed;
- ii. allow MOH entities to retain internally-generated funds to temporarily cover cash shortfalls and ensure continuity of operations.

Determining which option is more advantageous (or less disadvantageous) will require weighing the cost of allowing the retention of small idle balances in MOH-associated accounts (the cost of borrowing capital) or of allowing MOH entities to retain

internally-generated funds, with the anticipated hindrances to service delivery if cash rationing is put into effect (Pattanayak and Fainboim 2011).¹⁴

What can a ministry of health do?

The MOH will benefit from gaining an understanding of the MOF's rationale, priorities, and concerns related to cash management and consolidation. If the MOH then identifies a need for either of the two options (exemptions) above, it will be equipped to approach the MOF with a request that is based on evidence. The MOH's case will be stronger if it can also show that it is fostering good cash-management practices, such as improved cash plans, making more credible expenditure forecasts, submitting timely cash requests, and improving the monitoring of commitments and of payment schedules. The MOH can also strategically review its budget and decide on a schedule of "priority budget lines" which are most critical to ensuring continuity of service delivery. In the event of cash rationing, these budget lines should be fully funded first. If guidelines and rules are clear (that is, cuts are distributed according to a plan), this ensures that all of the MOH's budgetary units can effectively prepare for budget executions during shortfalls instead of being reactive.

14 The MOF can then optimize efficiency by allowing the MOH to manage the situation through these exemptions, which may be temporary. A decision to this effect should be grounded in data and evidence. The MOF should monitor the status of the TSA and cash management at the treasury, and can discontinue the exemption once the risk to service delivery is lessened.

What can a local government do?

A local government can advocate for intergovernmental transfers of funds that are predictable and adequate. This will enable the local government to have a

clearer and more accurate revenue position and allow that government to finance its service-delivery units in a predictable way. The recommended practice for health ministries mentioned above also applies at subnational levels.

Box 5.3:

Improving Regularity of Fund Flows: Illustrations of Good Practices in Several Countries

The Kyrgyz Republic's MOH ensured continued funding for priority health interventions, even during cash shortfalls, by increasing the share of the on-budget Mandatory Health Insurance Fund (MHIF)'s budget expenditures which were allocated to "protected line items." The MHIF disburses these line items at higher rates throughout the first 10 months of the fiscal year, and the MOF prioritizes these categories of expenditure in its cash-rationing system. Additionally, the MOF permits health service providers to carry unspent balances forward from one year to the next. Lao PDR's MOF strikes a careful balance between instructions for treasury to consolidate cash balances in the TSA, and allowing health facilities to maintain cash balances, optimizing implementation of the health budget. Ukraine's MOF updated its PFM regulations, abolishing controls on health facility inputs and granting health service providers managerial flexibility to reallocate expenditures during the process of budget execution. The MOF shifted to releasing funds to health facilities without ringfencing certain items; in turn, facilities markedly increased the speed of their financial operations, allowing for better execution rates. However, there was no systematically available information on facilities' actual spending compared to their planned spending.

5.4 Appropriateness and effectiveness of spending controls

Table 5.4

Potential actions for finance and health ministries, and local governments

Ministry of finance actions	Ministry of health actions	Local government actions
Create an enabling environment that balances control with the needs of health service providers Consider differentiated control policies (e.g., based on spending volume) Explore use of maturing-technology solutions	Engage with MOF on design of tailored control modalities Monitor and support application of control protocols Support purposeful use of FMIS or alternative systems within the MOH where appropriate Consolidate expenditure controls to reduce fragmentation from multiple financing sources	Provide supervision and oversight to health service providers Monitor and support application of control protocols Offer financial management capacity-building activities within local government and for health facilities

What can a ministry of finance do?

Ensuring that funds are spent within budget and as intended is an important function of a PFM system, and expenditure controls are an important tool for this purpose.

Designing and applying these controls rigidly across the budget can hinder budget execution, however. The MOF should acknowledge that a single set of rules (one-size-fits-all type controls) is unlikely to serve all types of expenditure transactions well. Rigid controls

may be appropriate for transactions constituting a large fiduciary risk such as bulk drug procurement, large transfers, or large capital outlays, but applying these to low-value transactions such as the procurement of basic commodities at the point of service delivery increases budget managers' workload, thus complicating budget execution with only very limited gains in risk-management. The MOF can facilitate health budget execution by designing and applying controls that are more targeted and sensitive to health service provider needs

(Piatti-Fünfkirchen, Hashim, and Farooq 2019).¹⁵ In addition, the MOF can proactively leverage financial technology innovations, such as the use of smart cards or mobile money to transfer funds to and monitor fund use by remotely-located providers, as a means of better applying controls (Piatti-Fünfkirchen, Hashim, and Farooq 2019, World Bank 2022).

What can a ministry of health do?

The MOH should keep track of the MOF's use of controls on MOH transactions; if the MOH finds that the MOF is applying controls to all transactions indiscriminately, the MOH should inform the MOF that this approach may not be appropriate and difficult to apply consistently to the budget. By identifying which transactions are high-risk, and health service provider needs, the MOH can aid the MOF to design more meaningful control protocols. The MOH can then support the application of those controls by using the government's FMIS purposefully and by making focused decisions about when and where it may be better to use a different type of expenditure management and accounting software than the FMIS.¹⁶ More broadly, the MOH can build trust by improving its capacity to manage the budget prudently. At health facility level, the MOH can reduce fragmentation that can result from multiple source financing, by streamlining the control protocols so that funds are subject to the same type of rules and controls regardless of funding

source – effectively becoming fungible at the point of use. The MOH can advocate to the MOF and to development partners for such an approach, regarding internally generated revenue streams and foreign assistance, respectively.

What can a local government do?

Local government authorities can play a central role in building basic financial management capacity at the provincial/ regional, district, or municipal levels. To the extent that the MOF delegates core functions including expenditures to health service providers, the district or municipal administration can provide financial management support directly to the service providers. Local government can also play a critical monitoring and supervisory role to ensure facilities manage resources effectively in a way that enables them to deliver essential services reliably.

15 In most countries, the expenditure profile is skewed such that a small number of high-value transactions makes up the majority of total spending; this in turn makes it possible to apply those controls in a risk-based and targeted way, by ensuring they are transacted through the government's FMIS (Hashim et al. 2019). Allowing for more context-appropriate controls will also facilitate more consistent application of appropriate controls (Hashim, Farooq, and Piatti-Fünfkirchen 2020).

16 The FMIS may be overly complex for use by primary care service providers; simpler financial information systems may be better suited to ensure basic reporting on and accountability for facility-level expenditures.



Box 5.4:

Adapting controls to health service needs: some emerging practices

In the Solomon Islands, the government took measures to reduce excessive internal administrative controls by allowing the Ministry of Health & Medical Services to process its own data and submit payment requisitions and documents to the Ministry of Finance and Treasury electronically. Pakistan's government introduced a "Green Corridor" to expedite low-value transactions. In Ethiopia, early in the COVID-19 pandemic, the government allocated flexible funds to the MOH, which was then free to use those funds to address identified needs and to make transfers to regional governments for activity implementation. One approach that appears not to have helped health budget execution was the Timor-Leste MOF's introduction and implementation of the Advance Payment Tracking (APT) tool, based on a newly developed electronic planning, monitoring, and evaluation system. The APT helps to monitor executing government entities' expenditure reporting and is intended to enable timely releases of next-period cash. As of 2022, the MOH, municipal health services, and referral hospitals had however found that the tool resulted in delays in the release of cash when they submitted their expenditure reports late.

5.5 Strengthening procurement and contract management

Table 5.5

Potential actions for finance and health ministries, and local governments

Ministry of finance actions	Ministry of health actions	Local government actions
Provide clarity on actors' roles and responsibilities, preventing conflicts of interest	Communicate with MOF on appropriateness of procurement processes in the health sector needs	Strengthen internal capacity, and that of service providers as relevant, for procurement and contract management
Facilitate an enabling legislative and information-technology environment	Delegate procurement authority closer to service providers, where appropriate	Delegate procurement authority closer to service providers, where appropriate
Configure FMIS to allow for processing of multiyear and framework contracts	Strengthen internal capacity to develop procurement plans, organize tenders, and appraise bids	Monitor and supervise service providers' procurement processes, as relevant
Facilitate sharing of data between FMIS and procurement system	Build internal technical capacity to manage contracts	

What can a ministry of finance do?

The MOF is responsible for providing clarity on roles and responsibilities in the procurement process – namely who manages procurement, who oversees implementation and monitors performance, who approves invoices, and who makes payments – in a way that avoids potential conflicts of interest. The MOF also needs to ensure that the FMIS system has the

functionality to process large, multiyear commitments; processing the latter within rather than outside of the system will enable the procurement process to respect budgetary and commitment controls, thus avoiding an accumulation of arrears and the crowding-out of other essential spending items (Hashim and Piatti-Fünfkirchen 2018). The MOF plays a key role in coordinating procurement, including by establishing and maintaining an e-procurement system. Such a system can accelerate and

make more transparent the major milestones in the procurement life cycle including planning, tendering, evaluation, awarding, and monitoring, thereby supporting budget execution.¹⁷

What can a ministry of health do?

The MOH plays a central role in procurement and contract management and should invest in its capacity to exercise this function well.

Once a government introduces framework or multiyear contracts, the MOH should invest in its technical capacity to manage contracts and its ability to continually prepare, negotiate, manage, evaluate, and conduct implementation reviews. To the extent that the MOH has a mandate to provide health services, the MOH can facilitate health service providers' work by delegating procurement authority to those providers and allowing them to make their own procurement decisions with the resources they have available. The central government can enable health facilities to procure essential medicines and commodities directly, by negotiating overarching framework agreements and multi-year contracts with vendors (Arney and Yadav 2014). Service providers can then select vendors from framework contracts that offer preferential rates, if available.¹⁸

What can a local government do?

The role local government plays in procurement can vary significantly depending on a country's administrative structure (including whether power is divided federally between a general government and subnational governments) and whether and how local government is involved in health service delivery.

In instances where procurement will affect a local government, that government can benefit from participating in the procurement committee. Technical capacity investments and adequate financial resource allocation are necessary for monitoring performance contracts. In contexts where local governments have authority for procurement of health-related products, those governments may be able to delegate procurement authority to health service providers located within their jurisdiction, with a view to expediting budget execution processes. To do this successfully, the local government may need to strengthen the service providers' capacity and monitor and supervise the providers' actions.

17 The MOF should ensure the e-procurement system exchanges data with the FMIS, which typically is limited to the procure-to-pay (P2P) cycle. By exchanging data, the two systems can jointly contribute to improved budget execution. For example, such data-sharing will make it possible to confirm the availability of a budget before issuing a contract, to synchronize the MOF's chart of accounts and master data on vendors, and to ensure the FMIS has information on commitments (Hashim, Farooq, and Piatti-Fünfkirchen 2020).

18 Particularly in LICs, development partners such as UNICEF can dominate health-sector-related procurement, especially for essential drugs and medical supplies and vaccines, with the process sometimes taking place outside of the government's procurement systems. The MOH can ensure the government maintains an accurate financial record of these outside transactions, by requiring partners to share information with the MOF which the latter can then capture in the government's FMIS (Piatti-Fünfkirchen et al. 2021a).

5.6 Strengthening monitoring systems and accountability

Table 5.6

Potential actions for finance and health ministries, and local governments

Ministry of finance actions	Ministry of health actions	Local government actions
Review expenditure data for adequacy Be transparent about any shortcomings in the expenditure data Explore options to strengthen adequacy of expenditure data Support use of FMIS where appropriate and enable information exchanges with other systems, as relevant Introduce performance monitoring framework to link budget to programmatic outputs	Ensure adequate expenditure reporting on the MOH's various financing sources and actors Facilitate data integration between information systems (ensure MOH's systems can integrate with the FMIS) Put in place data intelligence tools to identify links between spending and performance data, and enable joint analysis Facilitate attribution of results to spending, via appropriate institutional arrangements	Support health-related data collection, quality control, and performance monitoring Facilitate enabling institutional arrangements to link health facilities' results to budget execution processes

What can a ministry of finance do?

The MOF should ensure that expenditure reports for health and other sectors are comprehensive and relevant, and should make this spending data publicly available.¹⁹

Inadequate spending data can result in budget execution rates being calculated inaccurately. The MOF can assure comprehensiveness by processing spending through the government's FMIS as feasible, and by enabling data integration between financial information

¹⁹ The MOF should regularly review the data for the following core attributes and take action if it detects any shortcomings: (i) data provenance and integrity; (ii) comprehensiveness; (iii) usefulness; (iv) consistency; and (v) stability.

systems. Data also needs to be relevant to ensure that a useful evaluation can take place (for example, spending on primary care rather than only compliance on inputs). Secondly, the MOF can help set up systems to link expenditure with output data. One approach can be to introduce a health-focused program budget, with a consolidated performance monitoring framework for joint monitoring of financial and non-financial performance (Barroy, Blecher, and Lakin 2022).

What can a ministry of health do?

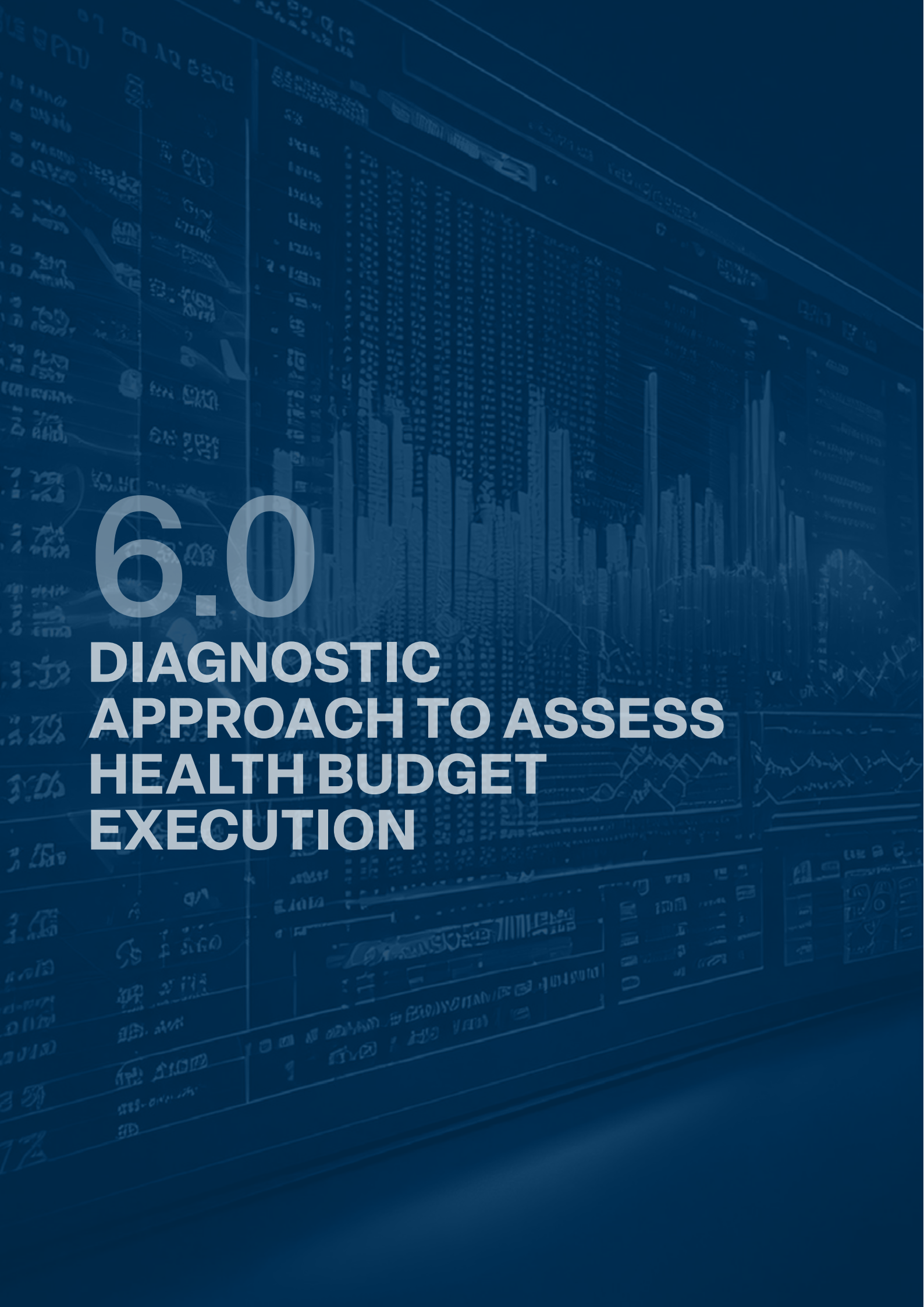
The MOH should help in producing adequate expenditure data, by ensuring that MOH systems capture and report back on spending from internally-generated funds, donor funds, or other entities such as a social health insurance agency. In doing so, the MOH should coordinate with the MOF to ensure that these data follow specified adequacy attributes and structure which will allow merging (or at least inter-operability) with other data managed by the MOF. Whether all of the MOH's expenditure information is to be captured in FMIS will vary by country and by specific type of expenditure. The MOH is also responsible for setting, monitoring, and collecting information on programmatic performance, including baseline data, targets, outputs, and outcomes, which can be triangulated with expenditure information, connecting spending with results.²⁰ Additionally, the MOH should ensure that its institutional arrangements facilitate close attribution between action and programmatic results in health facilities that have control

over their resources and can be held more directly accountable for the services they deliver (WHO 2022). Budget execution data should ultimately inform decision-making about future budget allocations.

What can a local government do?

The local government can play a central role in collecting operational performance data, ensuring data quality, and monitoring performance. To the extent that the local government has a mandate to oversee health services, it can help put in place institutional arrangements to ensure each health facility in its jurisdiction provides reports on both spending and programmatic performance (as opposed to having expenditures processed elsewhere).

20 The MOH can facilitate this triangulation by connecting its data warehouse with a data-intelligence platform (the final step in digitizing PFM), to support good use of those data (Hashim and Piatti-Fünfkirchen 2020).

The background is a blue-tinted image of a financial dashboard. It features multiple panels with data tables, bar charts, and line graphs. The text is overlaid on the left side of the dashboard.

6.0

DIAGNOSTIC APPROACH TO ASSESS HEALTH BUDGET EXECUTION

This section outlines a diagnostic approach for countries to assess their health budget execution performance. As previously highlighted, many LMICs face challenges in executing their health budgets. In the absence of clear guidelines, however, the specific nature and extent of these challenges, and their root causes, are often not fully assessed and documented. Having that information will make it easier for governments to develop effective policies to strengthen budget implementation in health. We therefore present a four-step diagnostic approach to enable the systematic assessment of health budget execution performance and to guide the identification of potential solutions.

6.1 Overview of the analytical approach

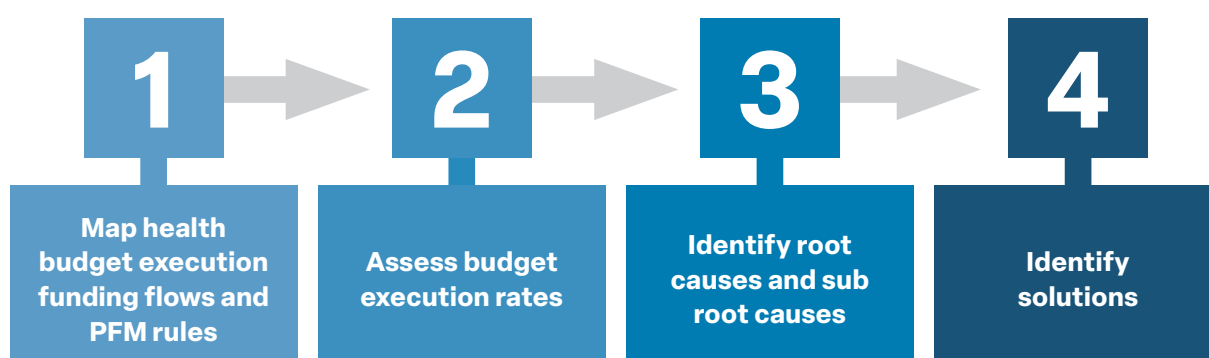
The analytical diagnostic comprises four main steps:

1. map of funding flows and review of PFM rules for each funding source in health;
2. provision of a detailed overview of budget execution rates;
3. identification of root causes and sub-root causes of bottlenecks in health budget execution;
4. identification of potential solutions

Each step is further delineated below. An example of country assessment questions is also provided in Appendix 3.

Figure 6.1

Four-step diagnostic approach for health budget execution assessment



6.2 Map health budget execution funding flows and PFM rules

Step 1

The first step consists of mapping health spending flows and identifying the associated PFM rules and characteristics for each source, with a focus on allocation and disbursement rules and processes. This exercise combines perspectives from health financing and PFM to provide an understanding of the ecosystem for health budget execution. The multiplicity and fragmentation of funding sources in the health sector can lead to each funding source having its own distinct allocation, disbursement, and reporting practices and rules. Therefore, it is crucial to consider these two aspects together as their relationship influences the effectiveness of budget execution in health.

In this process, the first sub-task is to identify and map the main sources of funding and how the funds flow to budget holders and service providers. This assessment should build on available guidance for mapping of funding flows in the health sector (see McIntyre and Kutzin in 2020) and draw on any existing mapping for the specific country. The second sub-task is to identify the PFM rules for each funding flow. As this is a less common task, countries may need to conduct a more in-depth review of PFM laws, budget documentation, and health-service purchasing literature and practice to identify the relevant rules and practices; and in turn to better understand how the country allocates, disburses, controls, and accounts for these specific funding flows.

6.3 Assessing health budget execution performance

Step 2

Budget execution performance is generally assessed through budget execution rates, where the level of actual spending is compared to the approved budget. This step begins with reviewing aggregate general budget rates. Next, is assessing health sector budget execution performance, which consists of calculating MoH budget execution rates, using the MOH's actual spending and its approved budget, to provide an initial quantitative understanding of health budget execution performance. The government can also collect data on other sectoral ministries, such as education, to benchmark the health sector's performance. Key sources of information include country-level budget law, expenditure reports, audits reports; Public Expenditure Reviews (PERs) and PEFA country assessment reports can represent an additional source of information.

Assessing additional indicators can provide a deeper understanding of health budget execution performance. Assessing the budget execution rate by quarter can provide information on disbursement timeliness. Funds that are predominantly released in the final months of the fiscal year can hamper budget execution if there is insufficient time for budget holders, who manage and account for expenditures, to fully execute their budgets. Examining execution by stage of expenditure (authorization, disbursement, payment) can

Table 6.1**Indicative indicators for measuring health budget execution performance**

Indicator	Measuring	Importance
Overall budget execution rate	Expenditure outturn relative to the original approved budget	Provides headline measure of overall budget execution; however, it can mask important aspects related to the quality of budget execution and does not provide information on sector spending.
Health budget execution rate	Extent to which the health sector budget expenditure outturn reflects the original approved budget	Provides information on the health sector-specific budget execution rate. This indicator can begin to unpack any health-specific budget execution challenges.
Health budget execution rate by quarter	Health expenditure outturn relative to approved budget by quarter	Provides information on the timing and alignment of health budget execution. Delayed releases can create a backlog of expenditure providing little time for spending units to fully execute their budget. If funds are not spent by the end of the fiscal year, then spending units often must return the unspent balance to finance. Unspent funds can incorrectly indicate that the sector was given too much funding.
Health budget execution rate by expenditure stage	Health budget execution rate by each stage from approval to payment	During budget execution, funding can be committed but not fully paid, leading to arrears. It is therefore important to understand the execution rate by expenditure stage, as it allows the identification of bottlenecks along the expenditure chain.
Health execution rate by budgetary programs (where they exist) or level of care (where available)	Health sector expenditure outturn by program or level of care relative to the original approved budget	This indicator is important for highlighting any differences in performance between programs or level of care, where the budget structure allows it. It can provide indication on what policies are prioritized/ringfenced or deprioritized during the spending process.
Health budget execution rate by economic type	Health sector expenditure outturn by economic classification relative to the original approved budget	Assessing aggregate health budget execution rates can mask variation in performance by economic type. Goods and services are generally underspent in health, while personnel remuneration is often well executed. Providing a comprehensive view of budget execution rates by economic items can shed light on those issues.

help in identifying bottlenecks in the expenditure chain. Last, disaggregating budget execution rates by expenditure categories (by economic items or programs, depending on the budget structure) can reveal the level of spending by major categories, unveiling specific deficiencies. Table 6.1 provides a list of indicators to guide a comprehensive assessment of budget execution performance in health.

6.4 Identifying root causes of health budget execution issues

Step 3

The third step involves using a matrix to systematically identify the root causes that impact health budget execution performance. By categorizing issues in a systematic way, the matrix facilitates the development of targeted solutions. The matrix's prepopulated categories (see Table 6.2, lefthand column) emerged from a comprehensive analysis of the literature on health budget execution in LMICs and from case studies of a selection of LICs and LMICs (see Section 4).

The first part of this step is to unpack the root causes of deviations from approved budget allocations (overspending or underspending) both by stage of the budget cycle (budget formulation, budget execution, and budget oversight) and by actor (MOH, MOF, and where relevant, local government). We recognize that the predefined categories (lefthand column) may not neatly fit with all countries' specificities, and that not all countries will face challenges in every category described; countries should populate the matrix to reflect their main issues.

The second part of this step is to unpack the subcauses for each identified root cause. This additional analysis is crucial because the same root cause may have different underlying subcauses, depending on the country. In turn, identifying the specific subcause(s) will make it easier to develop targeted policy actions. To illustrate, in one country the midyear deprioritization of certain health-sector expenditures may result from a sudden change in political priorities, while in another it may be due to insufficient revenues which constrain the budgetary space available for health. Though contributing to the same root cause, these different subcauses will likely require different policy responses. (See Appendix 3 for an illustration of the mapping of subcauses in one country).

Table 6.2

Matrix for identifying root causes and subcauses of health budget execution issues

	Root cause			Subcause		
	Finance	Health	Local government	Finance	Health	Local government
Budget formulation						
Credibility of the budget						
Budget structure and rules for allocation						
Budget execution						
Regularity and predictability of funding flows						
Appropriateness and effectiveness of spending protocols						
Effectiveness and efficiency of procurement and capital investment operations						
Budget oversight						
Relevance and quality of monitoring and accountability systems						
Inability to inform sector performance						

6.5 Identifying improvement measures to address causes of poor health budget execution

Step 4

Once finance, health, and (as relevant) local governments reach a shared understanding of health budget execution performance and of the root causes and subcauses of any challenges, these actors can define policy actions to address those challenges and strengthen budget execution. Table 6.3 provides a structured approach, in line with the previous analytical categories, for the MOF, MOH, and (as relevant) local government to identify concrete policy actions. Governments can also use the examples provided in Section 5 to guide the development of responses and policy actions tailored to their country's specific context and needs.

Using this simple, yet structured, diagnostic approach can aid the MOF, the MOH, and local governments in a given country to gain a detailed understanding of the current status of health budget execution, of specific challenges, and of the root and subcauses of those challenges. It also can equip them to identify relevant and targeted policy responses. Once actors go through this diagnostic process, they can coordinate with a wider group of stakeholders as relevant, both for input into and to gain buy-in or approval for the proposed solutions; in many cases this will be an iterative process. Once a policy response is agreed, the relevant actors can develop a clear implementation plan with specific actions, responsibilities, resources, goals, and timelines. This plan should be accompanied by a monitoring and evaluation component to track progress, adjust implementation as needed, and ensure accountability. Given that this will require input from and coordinated effort across finance, health, and different levels of government, establishing a coordination mechanism can ensure effective and efficient implementation and clarity throughout the process.

Table 6.3**Policy actions for strengthening health budget execution**

	Policy actions		
	Finance	Health	Local government
Budget formulation			
Credibility of the budget			
Budget structure and rules for allocation			
Budget execution			
Regularity and predictability of funding flows			
Appropriateness and effectiveness of spending protocols			
Effectiveness and efficiency of procurement and capital investment operations			
Budget oversight			
Relevance and quality of monitoring and accountability systems			
Inability to inform sector performance			

CONCLUSION

This report, a collaborative effort between the WHO and the World Bank, addresses the critical issue of health budget execution challenges in low- and middle-income countries. The data analysis shows that the mean level of health budget execution in 91 LICs, LMICs, and UMICs over the period 2010-2020 was 87 percent. The mean loss resulting from unspent budgets was significant and equal to what many countries spend on primary care. This represents a missed opportunity to leverage scarce resources and achieve key results in health.

The report identifies common factors hindering effective budget execution in health, pinpointing challenges on both the health and finance sides, as well as within local governments, across all stages of the budget cycle. The report highlights promising practices from specific countries, and offers policy options for MOFs, MOHs, and local governments to respond to budget execution challenges. Furthermore, the report proposes a diagnostic approach to systematically identify, analyze, and develop tailored responses to countries' context-specific health budget execution challenges. This approach, outlined in four steps, can help governments to gain a deeper understanding of their current situation, pinpoint specific challenges, and develop targeted policy options.

The findings underscore the critical need for a paradigm shift in how LMICs approach health budget execution. Simply focusing on expenditure management is insufficient.

Improving health budget execution requires a holistic approach, encompassing a strengthening of budget formulation and oversight functions, alongside implementing a more agile and responsive PFM system.

This necessitates proactive dialogue and collaboration among finance and health authorities, as well as with local governments in decentralized contexts. The report emphasizes that good health budget execution is not solely the responsibility of the MOH. Finance ministries must play a crucial role in making PFM systems more flexible and responsive to evolving population health needs.

The report's findings have far-reaching implications for achieving Universal Health Coverage. Effective and efficient health budget execution is not merely a technical issue; it is a fundamental pillar for ensuring equitable access to quality healthcare for all. LMIC governments can prioritize this issue on their agenda, recognizing its crucial role in advancing towards UHC.

The report serves as a call to action. It is time for LMICs to move beyond the status quo and embrace a more strategic and collaborative approach to health budget execution. By implementing the approach outlined in this report, governments can unlock the full potential of their health budgets, ensuring that every dollar invested translates into more tangible improvements in the health and well-being of their population.

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APPENDICIES

APPENDIX 1:

WHO and World Bank initiative on health budget execution

The work presented in this summary report involved a two-phase approach including data analysis, case studies, literature review, and consultations with experts.

The study's first phase (2017-2020) was an exploratory phase consisting of reviewing existing literature, identifying relations between budget execution and UHC, and providing a preliminary overview of trends in health budget execution (Piatti-Fünfkirchen, Barroy et al. 2021b).

The second phase (2021-2024) included an examination of the underlying causes of inefficiencies in countries' health budget execution in specific LMICs via 12 case studies conducted jointly by the WHO and the World Bank in 2021-2022, and a thorough analysis of health budget execution data from 91 LMICs for the period 2010-2020. The findings from the second phase informed the development of a matrix for identifying root causes and sub-causes of challenges in health budget execution.

Both phases included extensive consultation with experts in the framework of the Montreux Collaborative on Fiscal Space, Public Financial Management and Health Financing.²¹ This report summarizes the key findings from the two phases of work; in addition to providing visibility on health budget execution trends, the report proposes a matrix for countries to analyze their key bottlenecks in health budget execution and to help in identifying concrete solutions. For an overview of the ongoing joint WHO-World Bank initiative on this topic, see Appendix Box 1.1.

Appendix Box 1.1:

WHO and World Bank initiative on health budget execution: main goals

- › **Making challenges in health budget execution visible:** In the absence of consolidated global data on budget execution and LMICs' difficulties in accessing consistent national-level budget execution data, health budget execution performance has not been thoroughly assessed. This WHO-World Bank initiative aims to make health budget execution performance more visible to country and global leaders.
- › **Enabling a common understanding:** There is often limited understanding of the root causes of under- or overspending of budget allocations in health. The WHO-World Bank initiative aims to facilitate a common understanding of the challenges in health budget execution between national-level finance and health authorities, as well as with local governments where relevant, by providing an analytical framework to systematically identify driving factors of good/less optimal performance on both sides.
- › **Supporting the identification of solutions:** When finance and health authorities blame each other for inefficiencies in health budget execution, this can contribute to maintaining existing policies. The WHO-World Bank initiative aims to support tailored policy actions to address bottlenecks in health budget execution, by sharing good practices and systematically fostering the identification of solutions.

21 For the website of the Montreux Collaborative, see <https://www.pfm4health.net/>.

APPENDIX 2:

Data collection and analysis

At the time of analysis, there was a lack of publicly available global datasets on health budget execution rates. As such, the first step was to identify, extract, and consolidate country-level data from a range of sources. Data were drawn from the BOOST Open Budgets Portal²² and the Public Expenditure and Financial Accountability (PEFA) program's²³ country assessments, for countries classified by the World Bank as a low-income country (LIC), lower-middle-income country (LMIC) or upper-middle-income country (UMIC) over the years 2010 to 2020, inclusive. The dataset is made publicly available in conjunction with this report to enable further assessments.

Data on overall government and health-specific budget allocations and data on overall government and health-specific expenditure were extracted and consolidated from country-level datasets collected by the BOOST Initiative and from country-level assessment reports written by PEFA. Comparing the availability of country data from the two sources for the selected time frame led to the choice of the BOOST Open Budgets Portal, which offered datasets from a larger number of countries, as the primary data source. The BOOST Initiative's datasets were also more detailed, with granular-level data by budgetary component (for wages, goods, services and capital, for example) and budget execution data for other sectors such as education. In cases where both the BOOST Initiative and the PEFA program offered data for a given country, the BOOST Initiative data were selected. PEFA's data were selected in cases where data for a given

country were not available from BOOST. The resultant sample consists of 91 unique LMICs. See Appendix table 3.1 for the distribution of country data across income groups and WHO regions.

We developed a set of descriptive statistics for all sampled countries, disaggregated by World Bank income group and by WHO regional grouping, for the time-period 2010-2020. Time trends were reported as means with standard errors. Distributional analysis was assessed using boxplots reporting medians and interquartile ranges (IQRs). Analysis of health budget execution by budgetary component (wages, services, and capital) and against education relied on BOOST data given that such variables were unavailable from PEFA. All analysis was run in Stata version 14 (StataCorp, College Station, TX, USA, 2015).

Appendix table 1.1: Distribution of data

World Bank Income Group	No. of countries included in analysis
LIC	19
LMIC	38
UMIC	34
Total	91
WHO region	
African region	36
Region of the Americas	18
South-East Asia region	7
European region	13
Eastern Mediterranean region	7
Western Pacific region	10
Total	91

22 <https://www.worldbank.org/en/programs/boost-portal/about-the-portal>

23 <https://www.pefa.org/>

APPENDIX 3:

Country case studies

	Burkina Faso
Study period	2017-2021
Health budget execution performance	
- Overall absorption rate of the budget allocated to the Ministry of Health is 97.01 percent (2017-2021), ranges from 87 percent-110 percent against adjusted allocations; or 84-112 percent against original allocations. - The national universal health insurance fund (CNAMU) recorded a relatively low budget execution rate over the 2019-2021 period, approximately equivalent to 53.80 percent with a downward trend in the level of execution of its budget. - Broken down data at national level by economic classification also shows execution rates close to 100 percent for all categories ranging from 94.69 percent - 99.44 percent. Indicates good planning capacity, budgeting for health needs and resource consumption.	

Factors contributing to suboptimal health budget execution performance:		
Factor	Generic PFM factors	Health sector factors
Credibility of the budget	Inadequacies in revenue forecasting. Insufficient information on forecast budgets. Late communication of the ceilings.	- Planning based on unreliable data. - Poor preparation of the budget, based on historical budgets, general budgetary constraints and percentage increases compared to previous years. - Poor estimation of costs. - Insufficiency of information on the ceilings at the time of budget preparation. - Analyses like Public Expenditure Reviews, Medium Term Expenditure Frameworks, National Health Accounts are stated as not being utilised to make allocation decisions.

Burkina Faso continues next page...

Factors contributing to suboptimal health budget execution performance:

Factor	Generic PFM factors	Health sector factors
Budget structure and rules for budget allocation	Budget structure is rigid (input-based budgeting process although attempt to move to program based), lack of flexibility of budget holders in use of funds. Authorization for reallocation often takes time to be granted.	
Regularity and predictability of funding flows	Funding unpredictable. Mid-year reprioritization. Delay in releasing funds (also at LG level).	- Existence of multiple financing flows creates bottlenecks. - Ineffective cash management can lead to the delays in releasing the budget. - Delays in budget reallocation requests.
Appropriateness and effectiveness of spending protocols	High frequency of controls on the expenditure chain. Authorization modalities and cumbersome expenditure procedures.	- Low mastery of budget execution procedures. - Existence of multiple financing flows creates bottlenecks. - Non-compliance of deadlines by service providers or suppliers. - Hospitals face low quality of the supporting documents provided by the service providers, and large administrative burden. - Hospitals have payment arrears linked to non-compliance of execution deadlines, and late submission of supporting documents for expenses by suppliers after the deadline required for liquidation. - LG can delay payment of suppliers, and submitting documentation, also face challenges of administrative slowness. - LG execution challenges include connectivity issues for the Digital Spending Chain, meaning that stakeholders have to travel to the capital to manually enter data which loses them time. - Weak financial autonomy of the CNAMU. - The processing of requests is sometimes delayed, reducing the time available to carry out the initially planned activities.

Burkina Faso continues next page...

Factors contributing to suboptimal health budget execution performance:

Factor	Generic PFM factors	Health sector factors
Effectiveness and efficiency of procurement and capital investment operations	Cumbersome and complex nature of public procurement procedures also causes delays in the execution of activities and therefore limits the level of expenditure. Procurement processes are lengthy (average of three months) and pool of suppliers able to meet the department's requirements is apparently small. Excessive centralized administrative controls. Overly centralised procurement processes, all procurement processes, regardless of size, are managed by one department (DMP) for public procurement, regardless of the amount of the order. The time taken to execute orders is sometimes out of phase with that planned by management. This can lead to non-implementation.	➤ Procurement plans are not prepared on time and anticipated difficulties are not well considered. ➤ Internal disorganization of suppliers and service providers. ➤ Delay in awarding contracts. ➤ Limited knowledge and technical capacities of certain actors in the expenditure chain. ➤ Administrative burdens in public procurement. ➤ Procurement processes are not adapted to the specific needs of the CNAMU.
Relevance and quality of monitoring and accountability systems	Weak financial information systems. Lack of information on execution of the allocated amounts at LG level. Non-transmission of quarterly physical financial execution reports by town halls.	➤ Fragmented reporting, lack of data consolidation. ➤ Lack of internal (to health sector) processes for monitoring budget execution.



Cameroon

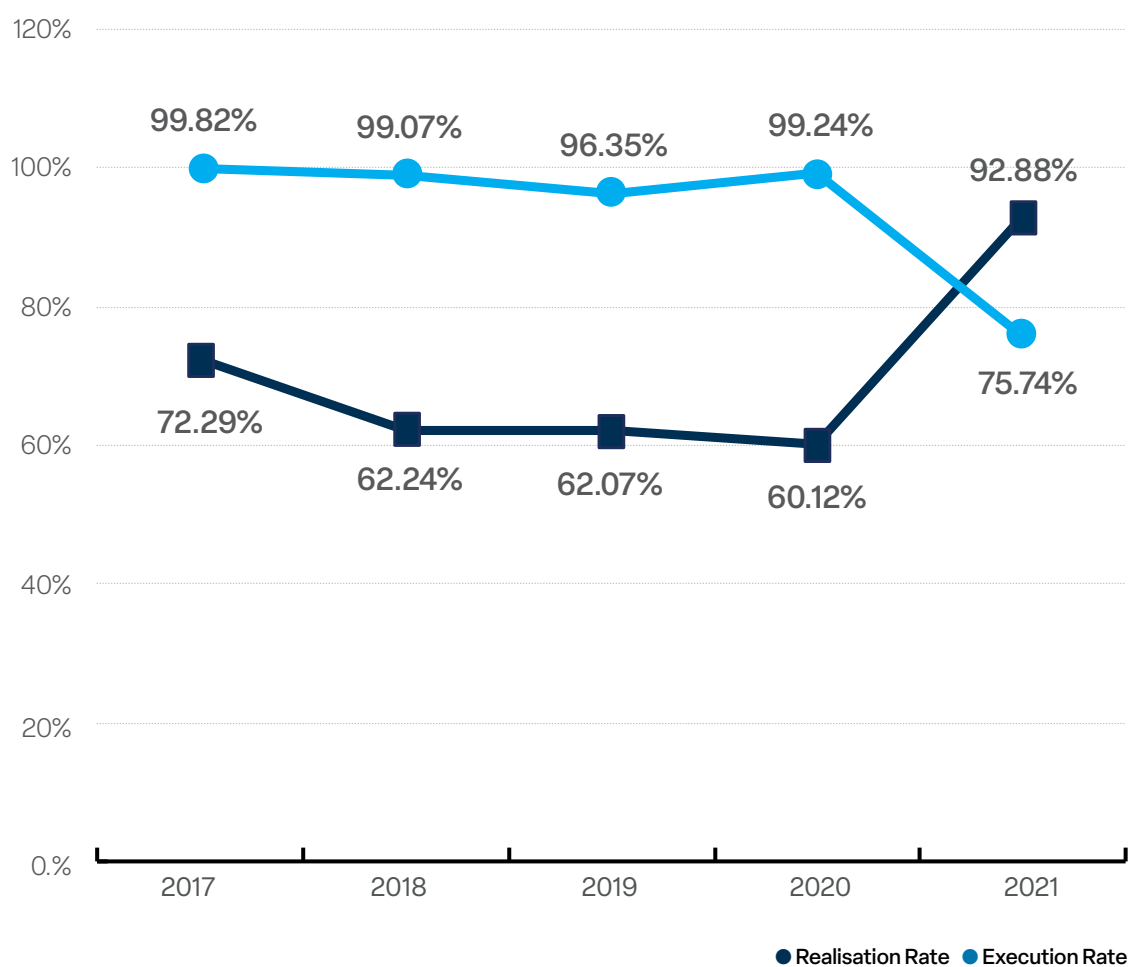
Study period

2018/19-2022/23

Health budget execution performance

- The execution rate was above 96 percent from 2017 until 2020. Decline in budget execution rate in 2021 to 75.76 percent.

Figure 2: Health budget execution trends



Factors contributing to suboptimal health budget execution

Factor	Generic PFM factors	Health sector factors
Credibility of the budget	Short time frame to develop budget of 15 days to draft and submit their comprehensive budgets. MTEF is rarely considered during budget negotiations and does not serve as a budget advocacy tool.	› Institutionalized separation of planning and budgeting processes. › Inadequate appropriation in implementing the operating budget. › Plans that contain unprioritized operations and budget request that can disregard the overall budgetary limits and limit national resources for which other sectors are also competing. › Budgets allocated to the various programs in the health sector do not reflect needs of the programs as expressed in their costed program plans. › Ministry of Public Health bases its budget allocations on the principle of institutional equality. This means that all health facilities rated at the same level of the health pyramid receive the same financial allocation, regardless of the population to be covered, general morbidity, and socioeconomic factors of the area. In part due to PROBMIS, the current budget management software.
Budget structure and rules for budget allocation	Transitioned to a program budget in 2007 but the budget still retains input-based budget characteristics.	› The budget has roughly 13,000 designated budget lines, which limits facility managers flexibility.
Regularity and predictability of funding flows	Weaknesses in the cash management systems include liquidity problems and inconsistent or insufficient disbursements of funds remain a challenge. Cash budgeting contributes to unanticipated, irregular, and decreased flow of funds, often accompanied by a funding boom towards the end of the fiscal year.	› Delays in fund release at health facility level; in addition health facilities received funding from different sources with different timelines and reporting systems which can create execution challenges.

Cameroon continues next page...

Factors contributing to suboptimal health budget execution

Factor	Generic PFM factors	Health sector factors
Appropriateness and effectiveness of spending protocols	Slow physical implementation of some joint financing projects due to delayed payment of counterpart funds.	› Cumbersome execution with multiple layers and numerous stakeholders. › Centralization of the PBF payment system ushered in new procedures and processes. Delays in PBF payment. › Administrative delays are also common due to the multiple steps involved in the award of contracts and the validation of contract files. Delays in the receipt of authorizations of expenditure remain a major challenge affecting budget execution within the MoPH. › Expenditures are made without abiding by the rules of commitment, liquidation, procurement, and payment.
Effectiveness and efficiency of procurement and capital investment operations	Insufficient execution of project contract procedures. Non-compliance with procurement plans. Lengthy time required to award public contracts.	› Inadequate programme management and operational plans. › Transfer delays due to cumbersome execution protocols and controls as a key issue. › There are numerous parties involved in the contract award process, which causes delays in the awarding of contracts and the signing of payment authorizations. This frustrates contractors and reduces their willingness to adhere to execution schedules, resulting in a low rate of investment budget consumption.
Relevance and quality of monitoring and accountability systems	No penalties imposed on state employees for mismanagement of public funds. Low level of reporting of financial data at all levels, deficiencies in the financial reporting system Absence of budget monitoring leads to under or over-spending, slow burn-rate, and lack of trust from donors.	› M&E not yet embedded in the operational culture of health sector actors.



Democratic Republic of Congo

Study period

2016-2020

Health budget execution performance

- › Over the period 2016 to 2020, the average execution rate of the general budget stood at 80 percent compared to 68.8 percent for health.
- › Low execution rates call into question the credibility of health budget forecasts, hampering the quality of activity planning and the effectiveness of public spending.
- › Over the same period, the gap excluding salaries averages 52.4 percent per year. By contrast, personnel expenses have a high budget execution rate each year, exceeding 100 percent. The average annual execution rate over the period 2016-2020 stands on average at 103 percent.

Factors contributing to suboptimal health budget execution performance:

Factor	Generic PFM factors	Health sector factors
Credibility of the budget	Lack of link between budgetary programming and ministry priorities. The sectoral envelopes notified by the Ministry of the Budget do not take into account priority needs and the Ministry of Health allocates resources by renewing budget lines from previous years. Budget preparation process is not very participatory. Lack of realism in the projection of revenues. Projection based on political objectives leading to an overestimation of revenues. The execution of which is difficult to achieve.	› Frequent overruns of personnel expenditure forecasts are observed each year. The envelope intended for personnel expenses is generally below real needs. › Failure to take into account recurring extra-budgetary expenditure for the same categories of expenditure. These expenses are never taken into account in the health budget despite their recurring nature. › Investment spending and spending on goods and services constitute extra-budgetary spending. E.g., The contributions for vaccines are paid by the Ministry of Finance without this being reflected in the health budget. › Failure to take health sector priorities into account in determining envelopes.

Democratic Republic of Congo continues next page...

Factors contributing to suboptimal health budget execution performance:

Factor	Generic PFM factors	Health sector factors
Credibility of the budget (continued)		› Failure to take into account all external financing in the health budget. › Sector priorities are developed after the budget preparation process, which does not allow them to be taken into account in the budget. › Weak budget preparation and poor cost estimates, leading to budget allocations misaligned with sector needs and making implementation complex.
Budget structure and rules for budget allocation		› The Minister of Health can decide to use resources for other needs without informing units.
Regularity and predictability of funding flows	Transfer delays due to cumbersome protocols and execution controls. At provincial level, reallocating these resources to new priorities which most of the time do not concern the health sector.	› Lack of communication and capacity in public finance management. Information on the amount and structure of the budget voted for the Ministry of Health is not known by all stakeholders in the ministry. The operational structures of the ministry do not have information on their allocation and the procedures to follow for the release of resources. › Hospitals have a specific budget based on their own resources and on the subsidy received from the State which is irregularly released.

Democratic Republic of Congo continues next page...

Factors contributing to suboptimal health budget execution performance:

Factor	Generic PFM factors	Health sector factors
Appropriateness and effectiveness of spending protocols	The strong concentration of budget execution of all ministries at the level of the Minister of Economy and Finance. The Ministry of Finance remains the sole authorizer of the state budget. Budget execution is governed by the budgetary commitment plan (PEB) and the cash flow plan (PTR) which are developed quarterly. These documents aim to prioritize the needs of ministries in relation to resource mobilization. Unfortunately, these documents are insufficiently used for the execution of expenditure. Operating expenses are not released for both health facilities and the structures of the Ministry of Health. Budgetary allocations are made each year in the finance law, but their execution is almost non-existent although these expenses have been included in the PEB.	» Frequent use of emergency procedures. This procedure, intended for specific and exceptional cases, has become the norm for budget execution to the detriment of the normal procedure. The consequence is the execution of extra-budgetary expenditure which does not correspond to the programmed priorities. » General non-compliance with expenditure controls. » Health actors who submit files late while the period for implementing activities is close. » The PEB being the reference for the execution of ministries' expenditure is often prepared late by the budget ministry. This delay also impacts the execution of the Ministry of Health's budget. » The lack of mastery of the procedures for executing donor resources which are sometimes different from national procedures but also the procedures differ from one donor to another. » The multiplicity of controls for non-wage operating costs in the normal procedure is not adapted to the objective of improving the level of health spending, especially to ensure the functioning of health care establishments. » Payment by the purchasing agency is hampered by delay in the provision of resources due to the delay in signing financing agreements due to the cumbersome nature of contractual procedures; the poor performance of providers who do not produce sufficient quantities of services to absorb the available funding, bad governance and fraud among providers which lead to a reduction in disbursements; delay in carrying out verification activities.

Democratic Republic of Congo continues next page...

Factors contributing to suboptimal health budget execution performance:

Factor	Generic PFM factors	Health sector factors
Effectiveness and efficiency of procurement and capital investment operations	The cumbersomeness of the procurement procedure with the different levels of control and intervention of several actors from different ministries does not facilitate the improvement of the level of use of resources. Non-prioritization of investments in PEBs adopted by the Ministry of the Budget. The investment amounts are always programmed in the budget but are rarely taken into account in the PEB for execution.	For bulk purchases of medicine, there are a fairly large number of controls which extend the delays and which can lead to a delay in providing patients with the necessary medications.

Tested solutions and good practices:

- Following an initiative dating from 2020, a tripartite Health-Budget-Finance committee was set up to monitor budget execution. In addition, the committee can propose corrective measures if necessary. The objective was to bring together around the same table the main actors in charge of executing the health budget to periodically analyze the execution situation in relation to forecasts and, if necessary, propose and monitor urgent actions to improvement.



Ethiopia

Study period

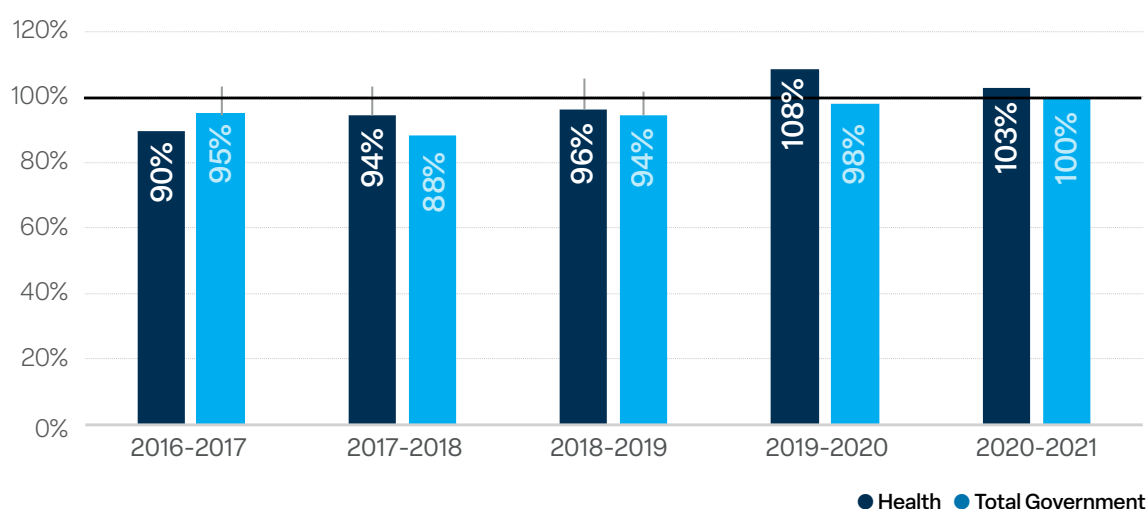
2016/17-2020/21

Health budget execution performance

2016/17-2020/21

- On average, between 2016/2017 and 2020/2021, 98 percent of the adjusted health budget was executed, while 95 percent of the overall government budget was executed.
- Apart from 2016/17, the health budget execution rate has been higher than the overall government budget execution rate, indicating that relative importance is given to the health budget in the overall context of public expenditures.
- The capital budget has weaker execution rates, ranging between 75 percent to 102 percent (average 90 percent) than the recurrent budget, which was executed at rates in between 97 percent to 115 percent (average 103 percent).
- Capital budgets are particularly under-executed at regional and woreda level.
- Recurrent expenditures are more predictable and therefore the execution rate is higher for both regions and federal government, and the adjustments mid-year through budget changes smaller.
- Regions are executing less of their budget compared with the federal government (apart from the year 2016/2017). This counts for both execution rates against the original budget, and the adjusted budget.
- During the Covid-19 outbreak, resources were allocated as grants to the Ministry of Health with unrestricted decision-making on how to spend it; this might have contributed to the over-execution of budget during that period.

Figure 1: Overall and health budget execution rates



Factors contributing to sub-optimal health budget execution performance:

Factor	Generic PFM factors	Health sector factors
Credibility of the budget		» Budgets from facilities tend to be inflated, assuming later cuts to it. If facilities get the budget approved in full by woredas (and even get revised further upwards by woredas) and regions, the facilities have unplanned resources available without the necessary capacities to implement the budget in full, reducing specifically capital budget execution rates. » External resources should be recorded and estimated more accurately and the process needs to be aligned with the government budgeting process. » The overall payment discipline of the government is high, with few arrears reported at the end of the fiscal year, although cash management has been cited as one of the challenges related to payment.
Budget structure and rules for budget allocation	Transfers within budget lines are possible with the approval of MoF – which can be a tedious process, limiting the flexible use of budget funds and ability to respond to needs.	
Regularity and predictability of funding flows	There have been instances of cash unavailability, leading to delays in transfers and some arrears due to not being able to pay invoices. Transfers to sub-national levels are announced on time and in a transparent way, using the approved resource allocation formula. However, actual disbursements can be up to three months late due to cash flow issues.	» Delays in approvals for budget reallocation requests, as well as funds and in-kind support coming through various channels, causing challenges with budget allocation and cash management and ultimately decreasing budget execution. » Budget releases reach facilities late, contributing to underutilization of budgets.

Ethiopia continues next page...

Factors contributing to sub-optimal health budget execution performance:

Factor	Generic PFM factors	Health sector factors
Appropriateness and effectiveness of spending protocols	Given the extent of the fiscal decentralization, there is very limited control from federal level over the actual utilization and onward transfers of block grants to regions. This could mean that regions allocate resources not in line with federal priorities, preferring their own regional priority list. This might lead to poor service delivery at woreda level.	› All three channels use different information systems to manage and track their expenditures, posing challenges for overall alignment to national priorities, as well as not fully enabling efficiency gains.
Effectiveness and efficiency of procurement and capital investment operations		› Capital budgets are particularly under-executed at regional and woreda level, with an average execution rate of 68% between 2013 and 2018, compared to 77% at the federal level during the same time. Some of the reasons might be that the cash for regional transfers is delayed, delaying budget implementation, or a lack of understanding, forward planning or other limitations for implementation of capital projects. › There are delays in processing procurements in addition to poor planning and implementation capacities at woreda level, reducing budget execution and causing room for inefficiencies.
Relevance and quality of monitoring and accountability systems	A high level of fiscal decentralization poses challenges in fully assessing the execution rates of health budgets, as well as their effectiveness and efficiency. Budget execution data is not publicly available, making accountability more difficult and decreasing transparency. Health stakeholders are involved in the annual planning session, but have limited insights into the budget execution progress.	› Currently, it is not possible to track program-related expenditures and execution rates across the health sector. › Unreliable, fragmented reporting systems across the different levels of government and channels of spending make it difficult to provide a comprehensive picture of budget execution. › Given that facilities are not linked with any of the financial management information system, it is difficult to get access to sufficient data on financial performance of health facilities. In case where audits conducted at facility level show mismanagement of funds – whether on purpose or by lack of better knowledge or capacity – there can be consequences and penalties for staff in charge.

Ethiopia continues next page...

- 】 Payroll accuracy is high, with change requests responded to within 48 hours if the budgeting entity is using the Integrated Financial Information System (iFMIS, implemented at federal level) and within weeks if outside of the iFMIS, but usually in time for the end-of-month payments.
- 】 The payment discipline of government is considered to be high in general, with arrears reported across government to be less than 1 percent of actual expenditures. This is partly due to a grace period at the end of the fiscal year to finish paying commitments, as well as regular reporting and rigorous commitment tracking of budget entities.
- 】 The budget calendar is adhered to and budgets for upcoming fiscal years, including details of transfers to sub-national levels, are communicated in time to regions and woredas. This helps the regions and woredas with their planning and commitment processes.



Kyrgyzstan

Study period

2016–2020

Health budget execution performance

- Between 2016 and 2020 the average budget execution rate has averaged approximately 93 percent compared to the revised budget, and 98 percent compared to the initially approved budget.
- The execution rate of the MOH budget compared to the revised budget has averaged 85.5 percent. The execution rate of the MHIF compared to the revised budget has averaged 94.7 percent.

Table 4. Health budget execution rates

Year	Approved Budget (MOH+MHIF)	Execution of the approved budget (MOH+MHIF)	Percent of the approved budget execution	Revised budget (MOH+MHIF)	Execution of the revised budget	Percent of the revised budget execution
2016	15659.5	14558.9	92.90%	15363.4	14558.9	94.70%
2017	15639.7	15421.3	98.60%	16598.5	15421.3	92.90%
2018	16568.4	16136.8	97.30%	17246	16136.8	93.50%
2019	17463.6	17075.9	97.70%	18470	17076	92.40%
2020	18283.5	19473.1	106.50%	21090.5	19473.1	92.30%

Factors contributing to suboptimal health budget execution performance:

Factor	Generic PFM factors	Health sector factors
Credibility of the budget	Poor quality revenue forecasts. Budget formulation based on historic expenditure. The Ministry of Finance (MoF) frequently changes the rules for budget preparation.	› Poor cost estimation and challenges with prioritization leads to discrepancies between budget allocations and actual policy costs and needs. › MoH, MHIF and health care organizations (HCOs) struggle with weak capacity and weak incentives among managers and finance staff to prepare realistic budgets.
Budget structure and rules for budget allocation	Although allocation is based on administrative and functional classification, during budget execution control is exercised using line-items control. Burdensome authorization procedures for re-allocation of funds across line-items during the fiscal year.	› Input-based norms continue to be used as the basis for budgeting by HCOs. › MoF and MHIF still require HCOs to submit budgets and cash plans broken down by economic classification for approval at the execution stage and for the purposes of “cash rationing” which is based on prioritization of line items.
Regularity and predictability of funding flows	As part of the cash management plan MoF withholds 30% of the budget allocation of non-protected line items (many non-wage operating costs and capital) in the first 10 months of the year. This leads to the accumulation of funds that must be transferred in the last months of the year. Late information about commitment ceilings. Mid-year budget re-allocations usually increase the budget but lead to lower execution rates.	› Fragmented funding allocations to HCOs for some programmes (notably the prescription drug budget) leads to a mix of unspent balances and unmet needs across different HCOs. › HCOs cannot absorb big bulk of funding arrived in last months of year effectively.
Appropriateness and effectiveness of spending protocols	Control of expenditure against budget is largely effective.	› Low autonomy of HCOs to manage budgets more efficiently and aligned with needs.

Kyrgyzstan continues next page...

Factors contributing to suboptimal health budget execution performance:

Factor	Generic PFM factors	Health sector factors
Effectiveness and efficiency of procurement and capital investment operations		› Lack of procurement specialists in the regional HCOs. › Delays in MoH's management of bulk procurement and capital procurement processes.
Relevance and quality of monitoring and accountability systems	Fragmented reporting systems make it difficult to provide a comprehensive picture of health budget execution. Long-standing weaknesses in personnel and payroll data and reporting. There is no accounting and reporting of budgets by programme. Onerous annual external audit of HCOs by the Chamber of Accounts makes managers cautious about re-allocating funds away from historic patterns and out-dated norms.	› Low capacity of HCO accounting staff in the regions. › Annual accounts and reports of HCOs, MHIF and MoH are not published.

Tested solutions and good practices:

- › Automation of the Central Treasury single account system beginning in 2014, all income and expense transaction are executed through the Single Treasury Account online.
- › Single line in the economic classification for MHIF's budget, which is intended to enable HCOs to reallocate funds at their discretion depending on priorities and needs. However, for purposes of budget execution and analysis, MoF and MHIF still require HCOs to submit for approval budgets and cash plans broken down by economic classification.
- › Other solutions that have improved flexibility is the reduction in the levels of detail used in the economic classification in the budget for MoH and its HCOs, removal of prior MoF controls on MHIF cash plan re-allocations and execution of payments to HCOs, providers can carry forward unspent balances at year end to the following year.
- › HCOs use one system for execution, accounting and reporting for all four sources of revenue.



Lao PDR

Study period

2015/16–2019

Health budget execution performance

- Overall budget credibility and reliability of the Lao PDR health budget is good. From fiscal year 2015/16 to 2019, the aggregate execution rate has ranged between 89 percent to 104 percent.
- The budget execution of capital expenditure is well executed. Total actual domestic capital spending was within six percent of plan for three of the four years analyzed, with the largest underspend being 13 percent in FY2015/16.

Table 3. Health budget execution rates

Fiscal Year	Classification	Execution Rate
2015/16	Ch. 60, 61: Salary, allowances and compensation	90%
	Ch. 62, 63, 64, 65, 66: Non-wage Recurrent Exp.	96%
	Ch. 67 Locally-funded Capital	87%
2017	Ch. 60, 61: Salary, allowances and compensation	99%
	Ch. 62, 63, 64, 65, 66: Non-wage Recurrent Exp.	98%
	Ch. 67 Locally-funded Capital	94%
2018	Ch. 60, 61: Salary, allowances and compensation	103%
	Ch. 62, 63, 64, 65, 66: Non-wage Recurrent Exp.	78%
	Ch. 67 Locally-funded Capital	96%
2019	Ch. 60, 61: Salary, allowances and compensation	98%
	Ch. 62, 63, 64, 65, 66: Non-wage Recurrent Exp.	103%
	Ch. 67 Locally-funded Capital	100%

Factors contributing to sub-optimal health budget execution performance:

Factor	Generic PFM factors	Health sector factors
Credibility of the budget	The budget preparation process for wages and salaries is not very precise, with worse estimation at provincial MoH compared to central MoH. Spending units prepare budgets for wages and salaries based on historical information plus a percentage increment. The submission is not linked to the number or type of positions, promotions, or allowances that it is intended to support.	Inaccurate revenue estimates reduce certainty over the total estimated health budget and complicate the work of planners. As revenues are a major source of funding for operations and maintenance this can delay the implementation of activities and lead to poor budget execution.
Budget structure and rules for budget allocation		- The implementation of National Health Insurance (NHI) has changed the budgeting processes for health facilities and impacted on budget execution. This change in budgeting practice has resulted in a poor allocation of funding. - Financial and health performance information is tied to budget allocation. Long processes and inaccuracies can lead to poor budget allocation and delayed cash disbursements which can impact on budget execution. - Disconnect between strategic policy goals and the existing administrative structure complicates the task of budgeting and oversight. For instance, it is difficult to determine which departments or divisions should receive additional funds to implement a strategic decision to increase the proportion of the health budget dedicated to preventative health care.

Lao PDR continues next page...

Factors contributing to sub-optimal health budget execution performance:

Factor	Generic PFM factors	Health sector factors
Regularity and predictability of funding flows	Cash shortages have pushed treasury to ration quarterly cash allocations, meaning that line ministries, such as MoH, have little certainty about quarterly cash-flow. With growing demands on revenues, MoH have reported quarterly cash disbursements are often delayed. Payments were once made at the beginning of the quarter but now are not received until the end of the quarter. Unpredictable and delayed disbursements reduce the time available and complicate the work of budget implementation.	- MoH manages large and complex revenue flows. The monitoring of revenue collection is complicated by the highly decentralized structure of MoH. Revenue forecasts and reports are consolidated at the district, provincial and then central levels. The lengthy consolidation process means revenue monitoring is delayed and forecasts are less accurate. - Health facilities have reported that NHI payments are increasingly unpredictable and delayed, even being received in the next fiscal year. Cash disbursement tracking reveals the extensive procedure required for cash disbursements to be released from the central NHIB department to health facilities. The long delays in receiving payment can shorten the timeframe for budget implementation. - Health facilities can maintain substantial cash balances, allowing for more independence from cash disbursements through the treasury system. These funds are only permitted to be used for non-wage recurrent expenditure. The large cash balances held by health facilities may partly explain the relatively good performance of non-wage recurrent expenditure.
Effectiveness and efficiency of procurement and capital investment operations	Explanation for the consistent over-execution of the capital budget at the local level can be found in the Amended Law on the State Budget 2006. Article 6 paragraphs 4 and 5 and article 32, paragraph 9 allows provincial authorities to allocate any revenues beyond approved plans to capital expenditures.	The Ministry of Health's low level of involvement during the implementation of capital projects is another area of concern. MoH involvement is essential to ensure that projects are aligned to strategic plans and that projects meet the needs of the ministry.

Lao PDR continues next page...

Factors contributing to sub-optimal health budget execution performance:

Factor	Generic PFM factors	Health sector factors
Relevance and quality of monitoring and accountability systems	The lack of transparency regarding public finances in Lao PDR is alarming. Of the few public documents that are produced by MoF, many are published late and lack detail.	› A minority of health budget units are using a financial management system approved by the Ministry of Finance. A survey of four provinces in 2018 found that 40 percent of provincial hospitals and half of health centres were not using the standard financial reporting forms required of all budget units. › Health facilities and many MoH departments do not have access to the GFIS financial management system. Health facilities, divisions and departments keep offline, and paper based financial records. Records must be consolidated at the district, provincial and the central level. The record keeping system is error prone while the many layers of consolidation lead to reporting delays.

Tested solutions and good practices:

- › The ring-fencing of the health budget is a factor that explains the good budget execution rates in the health sector.
- › While the Lao PDR is being forced to cut government spending due to increasing debt service costs, the government has committed to protecting the health budget. This assurance has helped MoH budgeting and has reduced the chance of in-year budget cuts.



Pakistan

Study period

2017/18-2020/21

Health budget execution performance

- Health budget execution rates are high (overall average of 90 percent); rates are even higher relative to the revised budget estimates (104 percent).
- Budget execution rates across provinces vary significantly, for example budget execution rates were 81.6 percent in Sindh and 82.6 percent KP in 2016/17, compared to over 100 percent in other provinces (i.e. Punjab 110 percent in 2016/17). Large spending variations within provinces likely highlight weaknesses in the budgeting process.
- Expenditure above 100 percent can be problematic and implies initial budgets did not anticipate policy changes.
- Expenditure-related data across facilities also indicate high variations in execution rates.

Factors contributing to sub-optimal health budget execution

Factor	Generic PFM factors	Health sector factors
Credibility of the budget	Overoptimistic provincial (own) resource estimates. Expenditure projections are based on uninformed revenue targets - evident from budget revisions.	- PHC facilities do not prepare their budgets, leads to misaligned demands and allocations. This greatly affects the day-to-day management of PHC health facilities. - PHC budget allocations are not based on type, size and catchment area (local demography) of the health facility. They are generated from historic allocation decisions. - Costing studies that inform budget estimates do not exist, as a result, budget allocations are not in line with needs.

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Factors contributing to sub-optimal health budget execution

Factor	Generic PFM factors	Health sector factors
Budget structure and rules for budget allocation		- PHC facilities are not spending units (cost centers) and do not have budget autonomy. - Vertical programs operate in parallel. They rely on different structures that can cause duplication (e.g., for supervision and M&E).
Regularity and predictability of funding flows		Insufficient, irrelevant and delayed resources compromise service quality (hygiene, access to basic utilities, availability of drugs, etc.). Reportedly, health facilities cannot pay utility bills, do basic repairs or maintenance, pay mobility allowances for vaccinators and LHWs, etc.
Appropriateness and effectiveness of spending protocols	Expenditure commitment controls not always followed. Excessive controls for operational expenses and inefficiencies prevail and applied to all transactions irrespective of the value. For example, a bill of PRs200 undergoes the same level of scrutiny as a bill of millions of rupees and involves duplicate internal controls of six different officers in two different entities.	- Total dependence of PHC health facilities on DHO or DAO for their spending (i.e. no autonomy) which can impact quality of healthcare delivery. - Facilities depend on DHOs that may ignore, delay, partially pay, etc. their requests. At the district level, the system is completely blind to the time it takes for DHOs to submit bills to the DAO. Claims can be parked at the DHO for months awaiting approval; as a result healthcare delivery in BHUs/RHCs can be severely affected.
Effectiveness and efficiency of procurement and capital investment operations		Audit findings noted multiple issues with procurement done at DHO, including non-compliance of controls, lack of transparency for tenders, purchasing medicine at a higher rates.

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Factors contributing to sub-optimal health budget execution

Factor	Generic PFM factors	Health sector factors
Relevance and quality of monitoring and accountability systems	Current CoA is not aligned with healthcare (referral) services and does not allow budget and expenditure recording of primary, secondary and tertiary healthcare (i.e. no such report can be generated). These inconsistencies make it hard to track budget allocation and expenditure at PHC level. Also impedes a smooth roll-out of performance-based budgeting that is mandatory at federal and provincial levels. There is no mechanism to record arrears.	› The current system does not establish clear lines of accountability between DHO/DAOs with financial responsibility and Medical Officers (MOs) with medical responsibility. › Weak transparency over budget at the facility level and tertiary facilities, including not maintaining proper books of accounts and non-availability of records.

Tested solutions and good practices:

- › Payroll is completely automated and salaries are paid regularly. Reportedly no arrears; no ghost workers.
- › The Federal government recently ratified a series of Acts to improve oversight of the health sector legislation. At tertiary level the Acts grant health facilities autonomous budget and expenditure management. Under these Acts, federal government transfers a single-line budget to these autonomous entities.
- › A remaining challenge is the quality of financial oversight and reporting inside MoNHSR&C.



Senegal

Study period

2017-2021

Health budget execution performance

- 】 The MSAS budget execution rate is at a satisfactory level (94.92 percent in 2020 and 95.34 percent in 2021, excluding personal budget).
- 】 From 2017-2021, health workforce spending was executed at 100 percent.
- 】 Spending on goods and services has been variable, due to unrelease of funds.
- 】 Capital expenditure, although experiencing a constant increase over time with the exception of the year 2020, has always seen much weaker execution.

Factors contributing to suboptimal health budget execution performance:

Factor	Generic PFM factors	Health sector factors
Credibility of the budget	The low participation of beneficiaries in the process. Failure to take into account the opinions of sectoral Ministries during negotiations with the Ministry of Finance and Budget (MFB). Low stakeholder ownership of the budget formulation process. Budget allocation poorly distributed and does not take into account objective criteria.	】 Weaknesses in planning by responsible units through the Annual Work Plan (PTAs). 】 Coordination problems between the State operating budget and the contribution of external partners within the framework of the PTAs.
Regularity and predictability of funding flows	Partial transfer of state budget resources/subsidies. Budgetary adjustments and budgetary cuts are often made. Administrative delays observed in State transfers (FDD, FECT, BCI) to LG, in addition scarcity of resources.	】 Delays are also often noted in the transfer of funds which is done in instalments for National Supply Pharmacy (PNA).

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Factors contributing to suboptimal health budget execution performance:

Factor	Generic PFM factors	Health sector factors
Appropriateness and effectiveness of spending protocols	Frequent shutdown of the SIGFIP system and delays in issuing payment orders.	- Incorrect budget allocation and non-compliance with the rate of consumption. - The absence of supporting documents or documents that do not comply with the expenditure procedure. - Non compliance of processes and authorisations. - Non-payment in full due to insufficient budget recorded on the line dedicated to its purpose. - The lack of mastery of changes e.g. new integrated financial management platform. - Lack of funds disbursement for certain programs. - Calculation errors on invoices or insufficiently detailed invoices. - Non-compliance with payment methods specific to each type of expense. - Rejection of authorisation and payment due to absence of correct documentation, errors in submitting reports, incorrect invoices.
Effectiveness and efficiency of procurement and capital investment operations	Cumbersome nature of procurement processes.	- Failure to comply with procurement plans. - The lack of justification for competitive bidding procedures.
Relevance and quality of monitoring and accountability systems		Absence of IT tools dedicated to monitoring and evaluation.

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Solomon Islands

Study period

2015-2020

Health budget execution performance

- Ministry of Health and Medical Services (MHMS) tends to spend most, if not more (104 percent in 2015) than its original Solomon Islands Government (SIG) domestic recurrent budget allocation each year. This is largely due to the large share being used for payroll and to the relatively non-discretionary nature of these expenditures.
- Because of the legal commitment related to paying payroll, expenditures are not restricted to the commitment process and are paid by MoFT automatically whether the budget is available or not.
- Budget execution rates for development partner recurrent expenditure are much lower due to poor integrated planning with MHMS systems, and delays in disbursement of cash from some DPs to MHMS.
- The SIG development budget execution rates were very low in 2015, 2016, 2017 and 2020 because of various bottlenecks and constraints, and potentially the Covid-19 pandemic in 2020.

Table Annex 1.2: Budget execution rates

Year	Budget Ledger	Original Budget Execution Rate
2015	SIG recurrent budget (L276)	104%
	DP recurrent budget (L376)	74%
	SIG development budget (L476)	34%
2016	SIG recurrent budget (L276)	105%
	DP recurrent budget (L376)	63%
	SIG development budget (L476)	14%
2017	SIG recurrent budget (L276)	103%
	DP recurrent budget (L376)	69%
	SIG development budget (L476)	36%
2018	SIG recurrent budget (L276)	94%
	DP recurrent budget (L376)	56%
	SIG development budget (L476)	211%
2019	SIG recurrent budget (L276)	88%
	DP recurrent budget (L376)	74%
	SIG development budget (L476)	101%
2020	SIG recurrent budget (L276)	101%
	DP recurrent budget (L376)	85%
	SIG development budget (L476)	34%

- SIG recurrent budget (L276) = domestic recurrent budget/expenditure.
- DP recurrent budget (L376) = Development partner (DP) recurrent budget/expenditure.
- SIG development budget (L476) = domestic development budget/expenditure.

Factors contributing to sub-optimal health budget execution performance:

Factor	Generic PFM factors	Health sector factors
Credibility of the budget	Development Partner recurrent budget revenues are often not realised. As such, budget execution rates are understated. MoFT budget submission timelines are often delayed. This has often resulted in MHMS not knowing its domestic budget ceilings until late in the process, leaving insufficient time to prepare the budget. This leads to confusion, poor planning and inevitably poor budget execution rates.	› MHMS has improved its budget preparations in regard to identifying the range of domestic and external funding to the health sector, however issues concerning adequate planning of these funds to operational plans still exist. Poor planning and scrutinisation of division operational plans by management impact poorly on execution rates. › A review by MHMS into the administration of employee allowances uncovered significant evidence of both under and overpaying of employees (2017 MHMS annual report); from 2018, the payroll and salaries budget was adjusted to a more realistic amount.
Budget structure and rules for budget allocation	Budgets are appropriated by division and by economic line item, and MHMS must formally request any changes to the original budget appropriation to MoFT if it wants to re-allocate budget. Re-allocations can only be done within a specific budget ledger.	

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Factors contributing to sub-optimal health budget execution performance:

Factor	Generic PFM factors	Health sector factors
Regularity and predictability of funding flows	Examples of delayed fund releases by MoFT are also reasons why outstanding arrears accumulate for MHMS. Budget cuts tend to impact development expenditures, i.e. a budget deficit in 2017 forced MoFT to cut back on some non-essential activities and caused the delay of payments which somewhat impacted the implementation of the 476 budgets for that year.	› Delays in disbursement of cash from some DPs to MHMS – both of which cause delay in activity implementation. › TB funding is managed centrally by the national divisions; poor planning of funding flows to the provincial areas has led to inefficient implementation of budget objectives. › Examples of delayed liquidation of funds impacting RMNCAH implementation of activities from provinces to the national program. › The SIG supplementary budget process can often be delayed, and as such, often additional development budget L476 funding made available to MHMS occurs late in the financial year where there is not sufficient time to implement the additional funds. › Examples of insufficient budget that requires divisions to do virements.
Appropriateness and effectiveness of spending protocols	MHMS is responsible for its budget execution, all transactions processed by MHMS are submitted to MoFT for processing – MoFT manages all the SIG bank accounts. Examples of delayed payment by MoFT are also reasons why outstanding arrears accumulate for MHMS.	› DP budget expenditure faces challenges due to complex payment processes, bureaucratic bottlenecks to access the funds, low absorption capacity to manage and implement these budgets. › Small MHMS teams that struggle to implement large DP budgets. › MHMS must wait for funds to be available in the DP bank account tied to the DP recurrent budget before they can start spending. There are ongoing complications and delays with accessing DP funds despite DPs theoretically being on-system. This can lead to low and/or late activity implementation. › The health sector is a crowded and complex financial space, with many domestic and external funding sources. There are a number of different funding modalities and timeframes for donor funding that are different to the SIG financial year. This creates significant pressure on the local staff and local system, as often there is a need to maintain parallel processes for each donor. As such, this can delay fund disbursement and implementation.

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Factors contributing to sub-optimal health budget execution performance:

Factor	Generic PFM factors	Health sector factors
Appropriateness and effectiveness of spending protocols		- Examples of delay in execution of recurrent budget due to noncompliance with financial processes. - Over-expenditure in payroll and staff benefits. - Spending limit for MHMS is set by MoFT, however weak controls and monitoring of funds flow and spending across by all Divisions has resulted in overspending or no payment made to supplier. - MHMS is a highly centralised organisation, but most health activities are required at the provincial level. Processing payments and allocating staffing, transport, logistic support centrally can be inefficient and lead to implementation delays, contributing to poor budget executions. As a result, funding and resources are often far removed from where they are needed.
Effectiveness and efficiency of procurement and capital investment operations	Very rigid and manual procurement systems lead to significant processing lead times for payments, which can be a bottleneck to implementation. The manual movement of paperwork between the MHMS and MoFT is inefficient and can lead to long delays when issues or discrepancies are found, and payments are rejected.	- The main challenges include the weak capacity of the infrastructure unit for planning and delivering infrastructure projects at the MHMS, and vacant positions within the unit. - Multi-year nature of infrastructure projects that do not align with annual budgets. Managing the annual 476 budget to fund infrastructure projects that span several years of implementation is a challenge for the MHMS. - National Referral Hospital procurement challenges lead to late initiation of procurement of specialized medical equipment's leading to low absorption and poor performance.
Relevance and quality of monitoring and accountability systems		- Some DPs require acquittals of funding tranches before new tranches can be released. Slow or inadequate reporting can prevent DP funds from being received, and as such, leads to low implementation rates. - Poor communication, unclear reporting lines, and lack of accountability at all levels of MHMS affect budget execution and service delivery negatively.



Timor Leste

Study period

2017-2021

Health budget execution performance

- › Average budget execution rate of health sector in Timor-Leste between 2017 and 2021 was 88 percent of the total government budget allocated to health and hit the lowest point of annual execution rate in 2021, 76 percent.
- › Overall, the average outstanding payment or arrears accumulated by the health sector institutions was below 2 percent, while in 2021 recorded 3.5 percent of arrears during the Covid-19 pandemic.
- › Between 2017 and 2021, on average, approximately 11 percent of the total government budget to health were not spent annually, remained in the non- obligated account which was returned to the treasury account at the year end.
- › The average annual health expenditure growth is 4 percent between 2017 and 2021, with fluctuation over the period.

Factors contributing to sub-optimal health budget execution performance

Factor	Generic PFM factors	Health sector factors
Credibility of the budget	Mixed approach to budget formulation using program-based budgeting and historic expenditure based allocation led to lack of clarity and confusion in budget allocation decisions. Direct budget allocation by the MoF to health sector institutions and off-balance budget plans by development partners contributes to unpredictable funding.	› Poor planning and budgeting practices have led to weak priority setting, poor cost estimations, and low budget executions of budgeted programs and budget line-items. › Weak understanding, technical knowledge and capacity in planning and budgeting, e.g., program-based budgeting. › Poor coordination among agencies within the health system. For example, the budget estimates are done by the MoH, while budget allocation decisions are made separately by the Autonomous Agency for the Purchase of Medical Supplies and Equipment.

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Factors contributing to sub-optimal health budget execution performance

Factor	Generic PFM factors	Health sector factors
Credibility of the budget (continued)	Time required for both national and sub-national governments to prepare programs' budget considered as limited and insufficient. Government entities have limited time to assess programs and define priorities to align with the proposed budget envelopes distributed by the MoF. Poor revenue forecast for the health sector.	Health facilities only need to fulfil the requirement to submit budget proposal on time. The baseline is largely derived from the previous year's spending, with less attention given to performance. This budget preparation practice has resulted in inefficient and ineffective budget execution and numerous in-year budget adjustments (i.e., virement).
Budget structure and rules for budget allocation	Poor budget execution in health sector is in part due to the ongoing PFM reform which includes implementing program-based budgeting. Technical staff do not have sufficient information on the reforms and updates undertaken at the national level. Rigid budget classification structures based on economic line-items. MoF plays dominant role in budget formulation and direct budget allocation authority. Complex, burdensome authorization procedures for re-allocation of funds across line-items in the cash plan.	- Ministry of Health still distributes budgets to municipal health services and referral hospitals using detailed line-items and they have no financial flexibility and autonomy to manage and control cash that was released to them. - Clarity on the implementation of program budgeting needs to be improved. - Fragmentation of activities and more than one entity being responsible for implementation has led to difficult program budget control and diminishes flexibility.

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Factors contributing to sub-optimal health budget execution performance

Factor	Generic PFM factors	Health sector factors
Regularity and predictability of funding flows	Late budget releases by MoF. Cumbersome authorization modalities and spending procedures. Quarterly expenditures limits of 25 percent of the total allocated budget.	- Health facilities rely on fund transfers from central MoH. However, health facilities do not receive budget transfers from central government in a timely manner and are uncertain if budget will be approved. - The quarterly basis cash release by the MoH to health service providers by line-items budget limits flexibility of finance managers in managing expenditures. - Health facilities delay the implementation of activities, reporting on expenditures and verification has frequently delayed next quarter cash release from the treasury. - Finance managers at the health facility levels have no authority to retain unspent cash. - Limited financial management capacity of health facility managers to manage multiple funding.
Appropriateness and effectiveness of spending protocols	Rigid budget execution rules and practice can lead to delays in spending the health budget. Payments are made through direct payment and through procurement process. Direct payment is straightforward and does not cause delays in spending as long as all required documents are available. No special arrangements to bypass any stages for approval. Therefore, any urgent payment requests cannot be accommodated immediately which can be problematic due to the nature of health sector activities.	- Key health programs budgets are managed and controlled directly by the Budget and Finance Management Directorate of MoH. They do not have direct involvement in the budget execution process until the release of the payments. - Multiple budget request approval stages at the health facility level contribute to the delay in budget execution.

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Factors contributing to sub-optimal health budget execution performance

Factor	Generic PFM factors	Health sector factors
Appropriateness and effectiveness of spending protocols (continued)	Next year's budget submitted to the national parliament at the same time as the closing of budget commitments has often caused payment delays. This has led to unanticipated payment arrears. Build-up of payments arrears has meant that previous year unpaid liabilities become a burden to current year budget execution.	
Effectiveness and efficiency of procurement and capital investment operations	Overly centralized administrative controls and cumbersome procedures. Payments made through procurement process take several days and even months to be released because the process must go through multiple stages and approvals.	▶ Limited technical knowledge and capacity on the management of procurement of goods and services. ▶ Lack of procurement specialists at the municipal health services and as well as at central MoH. ▶ 90-120 days for goods delivery term lead to stock out of drugs, medical equipment and supplies. ▶ Limited suppliers of goods and services available at the municipality level and low value tender for goods and services.
Relevance and quality of monitoring and accountability systems	Fragmented reporting systems make it difficult to provide a comprehensive picture of budget execution. Lack of regular internal and external audit and poor enforcement for follow-up.	▶ Lack of consolidated approach to expenditure and activity monitoring data. ▶ Paper-based expenditure reporting by the health facility finance managers, with potential errors for expenditure reporting. ▶ Low knowledge and capability of accounting and quality control staff both at the central and municipal level. ▶ Lack of regular dissemination and publication budget execution reports including internal audit reports are not shared with internal management team.

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Tested solutions and good practices

- › Transition to full program-based budgeting is in progress and on yearly basis the Ministry of Finance leads the rationalization of programs and budgets process during budget formulation and preparation process. As a result, the health sector institutions are left with two key budgetary programs – Health and Good Governance and Financial Management. The effect of such exercise has been significant as the government budget allocations to health can now be adequately tracked and monitored.
- › Delegation of Treasury functions from MoF to the Ministry of Health and Municipality Authorities and Administrations is enabled.
- › This decentralizing of Treasury functions is part of the PFM decentralization aimed at giving financial management autonomy to executing public entities, while the Ministry of Health strengthens and monitoring and verification system.



Uganda

Study period

2018/19- 2022/23

Health budget execution performance

Table 5. Health budget execution rates

Year	Approved Budget (Ugx Bns)	Supplementary Budget (Ugx Bns)	Revised Budget (Ugx Bns)	Absorption Rate	% Budget Released	% Budget Spent
FY2018/19	2310	56.5	2367	94%	80%	76%
FY2019/20	2590	125	2715	96%	89%	85%
FY2020/21	2789	324	3113	92%	81%	75%
FY2021/22	3331	397	3728	80%	95%	76%
FY2022/23	3685	318	4003	85%	82%	78%

Factors contributing to suboptimal health budget execution performance:

Factor	Generic PFM factors	Health sector factors
Credibility of the budget	Poor revenue estimates. Operational budget cuts, the central government has increasingly reduced recurrent expenditure to save funds for increasing the development budget. Regular supplementary budgets. Weak linkage between resource allocation and results.	» The government uses a resource allocation formula to allocate grants to health facilities which does not take into consideration all factors. » The health sector planning is inadequate, and prioritization and sequencing of health projects remain challenging, creates budget execution challenges. Delays in project implementation due to poor planning lead to time and cost overruns. In addition, there is a tendency to implement new work plans before completing the old ones leading to over commitments and arrears.

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Factors contributing to suboptimal health budget execution performance:

Factor	Generic PFM factors	Health sector factors
Credibility of the budget (continued)		› Lack of equity and discretion of resources allocated to LGs in the hard-to-reach areas. › The local governments cannot influence central government funding allocations to the health facilities under their jurisdiction, which compromises transparency in the budget process. › Issues of allocative efficiency due to poor planning and prioritization.
Budget structure and rules for budget allocation	Budgets that are insufficiently funded compromise the quality of services due to the rigidity (10% cap) in re-allocation between budget lines evident in the PFM Act 2015. More capacity and resources required for the PBB reform.	› Fragmentation means that providers can use funds from specific sources for certain items but cannot use all funds for all line items, leading to inefficiency in budget execution.
Regularity and predictability of funding flows	Delays between when the MoFPED issues the release paper and when the local government receives the funds. As a result, local governments often have to reconcile the paper release and amounts transferred to the local government bank account. Sometimes, the payments received are inconsistent with the release schedule.	› Failure or delay in release funds to pay suppliers of medicines and pharmaceutical supplies, lead to disruptions in service delivery and curtail implementation of health sector workplan.
Appropriateness and effectiveness of spending protocols	The rigidities in spending protocols create service quality issues since health providers spend on certain pre-defined items for budget compliance reasons, thereby undermining the quality of services.	› Each financing source often has its spending protocols, rendering the execution environment for these service providers fragmented. › Poor pace of implementation and absorption for donor funded projects. › Need to connect all hospitals to IFMS, PBS and other platforms.

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Factors contributing to suboptimal health budget execution performance:

Factor	Generic PFM factors	Health sector factors
Effectiveness and efficiency of procurement and capital investment operations	Procurement delays.	» The health sector experienced delayed procurements due to poor procurement planning. Procurement delays coupled with the inadequate capacity of contractors affected the timely completion of planned works and drug supply. » Operational delays in the procurement process lead to stockouts of essential drugs.
Relevance and quality of monitoring and accountability systems	Weak enforcement of accountability and transparency in service delivery.	» Excessive financial management requirements. » Health centers II and I do not employ full-time staff to support health care workers in financial management, especially in accounting and financial reporting. The health care providers sometimes double as administrators at the health facilities and are consumed in accounting tasks where they do not have the requisite knowledge. » Late submission of accountabilities by LGs.
Other	Weak internet connectivity and bandwidth in some local governments.	

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Tested solutions and good practices:

- › PFMA Reforms include strengthening the legal framework for budget preparation; approval; execution; accountability of public, strengthening linkage of annual budgets to long-term development agenda, introduction of PBB to improve synergies among MDAs, automation of PFM systems such as the PBS, IFMS, EGP, IBP, OTIMs.
- › The government of Uganda introduced a Treasury Single Account to improve the predictability of funds to Ministries, departments and agencies as well as sub-national governments for budget execution.
- › For the health sector the IFMS was rolled out at the Ministry of Health, Local governments, Hospitals and Health Centre IV. The expenditure controls at Hospitals and HC IV have been strengthened through the use of electronic cash payments, enhanced system security, fixing of cash withdrawal limits and streamlined approval and authorisation processes. However, the plan to take the integrated financial management system down to lower local governments and health centers is not yet approved.
- › To strengthen budget execution controls, the government payroll was automated and decentralized. The automation of the payroll system improved expenditure controls, accuracy and reliability of budgeting for staff salaries and wages, reduced leakage, and reduced errors in payroll systems.



Ukraine

Study period

2015-2021

Health budget execution performance

- Overall health budget execution rates tend to be less than 5 percent lower than adjusted annual plans in all years except post-crisis 2015 and 2020.
- The central government adjusted plans tended to be relatively more accurate than sub-national budgets in 2016-2019 (with health budget execution rates at the central level 2 to 3 percent lower than adjusted plans, while sub-national government spending was 3 to 6 percent lower).
- Within the the Programme of Medical Guarantees (benefit package) (PMG), execution rates across types of services are more variable, reflecting the more challenging task of forecasting expenditure based on new payment methods and the new degree of flexibility to adjust plans within the integrated PMG budget.

Health budget execution rates (2015-2021)

	2015	2016	2017	2018	2019	2020	2021
Consolidated budget	-6%	-4%	-5%	-4%	-3%	-6%	-3%
Central budget	-10%	-3%	-3%	-3%	-2%	-6%	-3%
SNG budget	-5%	-5%	-6%	-4%	-3%	-5%	-5%

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Factors contributing to sub-optimal health budget execution performance

Factor	Generic PFM factors	Health sector factors
Credibility of the budget	Budget allocations to the MoH and PMG are below the level required to meet current policy commitments.	- Challenges in forecasting components of the PMG (e.g. Affordable Medicines Programme). - Methodology for service costing and pricing is not sufficiently transparent. Costing does not cover expenses for PMG services that patients pay out of pocket (though most PMG services are theoretically free to patients).
Budget structure and rules for budget allocation	PMG budget appropriated by the legislature is a single programme line allocation, which allows for flexibility. Re-allocation across programmes is possible via executive decisions however, this creates variability in re-allocations and execution rates.	- New legal status of the providers provides more autonomy, moving away from the rigid input-based spending norms (such as staff numbers per bed and salary schedules) and controls to output-based contracts and ex-post reporting and monitoring of inputs. - There is a lack of methodology for allocating PMG budgets across regions and services in line with patient needs. Allocations for many services are driven by supply, not need.
Regularity and predictability of funding flows	Precedents of reducing PMG budgets via executive CabMin decisions to cover unexpected cash needs of the wider Government create a risk of unpredictability.	Facilities have their own bank account which has led to some cash management issues, e.g. healthcare providers deposit revenues in commercial banks and hold balances to earn interest.

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Factors contributing to sub-optimal health budget execution performance

Factor	Generic PFM factors	Health sector factors
Appropriateness and effectiveness of spending protocols	Highly centralized and control oriented PFM system with a number of stages, approvals and rules. This prioritises discipline often at the cost of efficiency and result orientation. The intricate process of Treasury cross-checks to ensure that commitments remain within appropriations, expenditure ceilings, and available budget funds, makes the controls highly effective.	- Annual processes for approving the PMG are complex, onerous, leading to very short time frames for contracting providers and provider uncertainty about future revenues until the beginning of the budget year or later. - Facilities have almost complete flexibility in managing their budgets from the National Health Service of Ukraine (NHSU) revenues and revenues from paid services in theory, though sub-national government owners influence or limit changes to resource allocation in practice.
Relevance and quality of monitoring and accountability systems	Programme budget indicators are formulated and reported but need to be more results-oriented. Programme indicators are not yet used actively to drive accountability or analysed and used to inform future budget allocations. Auditors do not fully understand the health reform and lack technical capacity. Audits have also become increasingly punitive. This has demotivated providers to exercise their new managerial flexibility and spending efficiency.	- Since introduction of provider autonomy and implementation of PMG transparency and accountability of facilities has decreased. Payments to autonomous providers are accounted as “transfers to organizations” without the breakdown by economic classification lines. - There is no publicly available information on actual amounts compared to the planned spending at the facility level. Facilities report expenditure quarterly to NHSU by inputs, but these reports are not audited and may differ from audited accounts available to sub-national governments.

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Tested solutions

- › Public financial management and public administration reform occurred before health financing reform, creating a platform of systems and capacities that facilitated implementation of health financing reform. These included an automated Treasury system, programme budgeting, and performance agreements with directors of executive agencies such as NHSU.
- › Health financing reform - which included the shift to autonomous public providers, the introduction of the PMG benefit package and output-oriented payment of providers - provided more flexibility and autonomy. Regulations that controlled health facility inputs were abolished. Previously, providers had little managerial flexibility to reallocate expenditures during budget execution. Amending the budgets was a bulky exercise requiring a complete repetition of the drafting and approval process. Overall, the speed of financial operations at the facility level following the health financing reform has increased rapidly.
- › Health care is one of only two areas where key spending units are allowed, in specified cases, to assume expenditure commitments stretching beyond one year (the second area being long-term contracts on energy-saving investment). Such commitments are allowed for up to three years on centralized medicines and medical goods purchases.
- › Re-allocations within the PMG are linked to adjustments in the forecast utilization of services, particularly within the Affordable Medicines Program (AMP). For instance, the allocation for the AMP was reduced following a forecast estimating lower utilization and not the deprioritization of this program.

APPENDIX 4:

Mapping Sub-Causes of Poor Budget Execution in Health – An Illustration from Uganda

Phase: Budget formulation	
Causes	Sub-Causes
Finance	
Inaccurate revenue projections	Poor macroeconomic and fiscal forecasting models. Insufficient technical capacity. Unanticipated revenue shocks. Deliberate overestimation of revenue.
Short timeframe to develop budget development	Poor communication of budget timetable. PFM laws provide insufficient time. Non-compliance with the established budget timetable.
Limited communication of budget ceilings to health	Poor communication between finance and health. Non-compliance with the PFM regulations.
Slow transition to flexible budget structure	Capacity issue for defining appropriate programme budget structure. Political economy considerations. Resistance to change.
Inadequate sector-specific guidance on PBB design and operationalization	PFM and PBB reforms led by finance with minimal understanding of health spending issues.
Limitations on budget re-allocations	Rigid economic item formulation. Lack of flexibility in PFM law for re-allocating budgets across lines.

Phase: Budget formulation

Causes	Sub-Causes
Health	
Overuse of historical budgeting	Short time frame to produce budget estimates. Weak technical capacity and inappropriate tools. Historical preference for this methodology.
Overreliance on input based planning	Historical preference for this methodology. Limited technical capacity and understanding of formulation.
Inaccurate cost estimation	Weak technical capacity and inappropriate tools. Uncertainty in health needs. Underpinning economic assumptions are weak. Deliberate underestimation of costs.
Poor priority setting and limited alignment of budget proposals and health needs	Lack of communication or coordination between health and finance authorities. Lack of inclusive budgeting process (finance-led only).
Continued use of input-based budgeting	Sector resistance to adopting new approach. History of input based planning.
Inappropriate application of Ministry of Finance guidance	Lack of communication or coordination between health and finance authorities. Limited technical capacity to implement new guidance. Lack of adequate direction or procedures for implementing new guidance.
No alignment between allocation and implementation responsibility	Lack of communication or coordination between health and finance authorities. Lack of inclusive budgeting process (finance-led only).

Phase: Budget execution

Causes	Sub-Causes
Finance	
Systematic delays and incomplete budget releases	Poor cash flow forecasting and ineffective cash balance management. Procedures for the release of appropriations is inadequate. Multiple bank accounts. Inadequate IT systems.
Cash rationing due to lower-than-expected revenues	Unanticipated revenue shocks. Deliberate overestimation of revenue. Weak cash management systems.
In year de-prioritization of health budget	Change in political priorities. Weak priority setting. Overspending in other sectors or economic items. Insufficient funding.
Cumbersome authorization and spending procedures	Lengthy and numerous ex-ante controls. Authorization required from multiple layers of government. Excessive ex-ante controls for small value transactions. Spending and release of funds over reliant on finance authorization.
Excessive centralized control (no delegation of authority)	Disbursement rules established by finance authorities. Lack of coordination or integrated feedback from local government or health authorities. Political preference for centralized control . Capacity constraints at local government and health authorities level.
Multi-layered and over-centralized procurement rules	Cumbersome and lengthy protocols. Multiple layers of government involved.
Lack of appropriate procurement protocols and process for health-related products	Insufficient coordination and communication between health and finance.

Phase: Budget execution

Causes	Sub-Causes
Health	
Delays in cash requests from health	Inadequate communication between health and finance. Poor management of commitments. Weak financial management capacity.
Fragmented health financing flows and inconsistent disbursement schedule including external resources	Multiple health financing flows with own spending modalities. Uncoordinated and disjointed health financing mechanisms.
Lack of autonomy for service providers	Limited ability to receive and manage funds. Lack of recognition in chart of accounts. Rigidities in the use of funds.
Limited financial management capacity	Weak technical capacity to undertake complex financial management systems. Insufficient financial management human resources.
Constrained ability of service providers to develop and implement procurement plans	Weak and insufficient technical capacity to undertake procurement. Insufficient time to execute all requirements.
Inefficient operationalization of capital investment by budget holders	Weak and insufficient technical capacity to undertake capital. Insufficient time to execute all requirements.

Phase: Budget oversight

Causes	Sub-Causes
Finance	
Limited capacity to produce comprehensive execution reports	Absence of guidance or mechanism to undertake monitoring. Insufficient clarity on responsibilities regarding monitoring and reporting. Absence of oversight requirements for health authorities to collect data. Insufficient time or tight deadlines to execute monitoring or oversight.
Limited availability and accessibility of FMIS	Partial roll-out of IFMIS. Limited IT infrastructure. Insufficient and weak technical capacity. Lack of accountability or enforcement for sidestepping FMIS.
Emphasis on input-based accountability	Systems still use input based approach. Preference for inputs approach.
Limited usefulness for health of execution reports	Inadequate or inappropriate data collected that does not align with health outcomes or priorities defined by the budget. Lack of process to translate collected data to actionable solution to improve health budget execution Limited coordination and communication between health, finance and local government.
Insufficient emphasis on sector performance	Absence of feedback mechanism for sector performance. Limited technical capacity to undertake sector performance assessment. Lack of performance metrics and goals. Resistance to change, leadership does not emphasize performance monitoring and feedback mechanism.

Phase: Budget oversight

Causes	Sub-Causes
Health	
Cumbersome financial monitoring and reporting requirements	Excessive or redundant procedures . Manual process for monitoring reporting, and accounting.
Lack of data consolidation for monitoring financial and non-financial performance	Lack of management systems for consolidating different data. Cumbersome, duplicative reporting mechanisms. Insufficient financial management capacity.
Inability to triangulate expenditure with outcomes data	Lack of management systems for consolidating different data. Cumbersome, duplicative reporting mechanisms. Insufficient financial management capacity.

ECO-AUDIT

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BUDGET EXECUTION IN HEALTH: FROM BOTTLENECKS TO SOLUTIONS